

Tango Therapeutics

It Takes Two to Make a Revolution: Initiate at OW

We initiate coverage on TNGX with an OW rating and a Dec-2026 price target of \$46. Following its monotherapy PoC last year with lead PRMT5i vopimetostat, Tango is leading the way in PDAC and potentially NSCLC through its combo trials with RAS inhibitors daraxonrasib and zoldonrasib. The first look at the combo data earlier this year with RVMD's data showed impressive response previously unheard of in the 2L+ PDAC setting with a 92% ORR and 6-month PFS rate of 90% in a setting where, until recently, median OS was ~6 months and ORR of 10%-15%.

With combo data in hand, a Ph 3 trial in 1L PDAC is in the works, pending regulatory feedback, to gain a lead over competition, which is also collaborating with RASi players. Tango is also hedging its bets in this setting with a recently announced collaboration with ERAS. With the pipeline now streamlined to focus solely on MTAP-deleted cancers, TNG456, Tango's CNS-penetrant PRMT5i is in the clinic targeting glioblastoma, which is another difficult to treat indication, and if monotherapy PoC is shown later this year, a combo with LLY's CDK4/6i will follow. **Net-net**, Tango remains true to its name, with multiple disruptive combo tx options in the works targeting MTAP-deleted cancers. **Initiate OW.**

- **PRMT5i + RASi combos have the potential to change PDAC treatment.** Data for the vopi combo with RVMD's data showing a 92% ORR in 2L+ PDAC is a meaningful step above dara's 30%-35% recently seen in Ph 3 RASolute 302 which is considered a revolution in 2L PDAC. While still early, excitement is meaningful as ~22% of the ~66K annual US PDAC cases have MTAP-deletion driving Tango to plan a Ph 3 1L PDAC trial to be finalized in 2H.
- **Tango isn't putting all its eggs in one basket and is diversifying across the RASi space.** The vopi combo with RVMD's data and zoldon is leading the way in this new combo paradigm, but neither intends to be exclusive as this new space shapes up. As RVMD is also collaborating with competitor BMY's navlimetostat, Tango has entered a collaboration with ERAS (covered by Anupam Rama), RVMD's lead competitor, hedging their bets in the RASi space to ensure vopi will be in the lead as it plays out.
- **GBM through TNG456 has strong preclinical rationale with combo next step.** Monotherapy data is expected this year from the Ph 1/2 trial in GBM (~45% delMTAP) with brain-penetrant TNG456. Initial PoC will trigger part 2 combining TNG456 with abemaciclib, LLY's CDK4/6i. This could open up another combo front for a non-overlapping MOA in a setting that is ripe for new treatment options, which we believe isn't getting value from investors just yet.
- **DCF-SOTP valuation:** Our Dec-2026 PT of \$46 leverages our DCF-based SOTP model. We assign stand-alone values for vopi in 1L PDAC and 2L NSCLC and for TNG456 in GBM. We assume vopi's peak WW sales of \$5.4bn and ~\$600mn for PDAC and NSCLC, respectively. Risks associated with our rating and estimates include: competitive risks in the tx landscape, cash needed to reach the next set of major inflection point, and clinical and regulatory risks that are typical of Biotech cos. at this stage in development.

Initiation Overweight

TNGX, TNGX US
Price (14 Jul 26):\$29.65
Price Target (Dec-26):\$46.00



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Quarterly Forecasts (FYE Dec)

Adj. EPS (\$)	2025A	2026E	2027E
Q1	(0.36)	(0.32)A	
Q2	(0.35)	(0.31)	
Q3	0.12	(0.30)	
Q4	(0.33)	(0.31)	
FY	(0.88)	(1.30)	(1.24)

Style Exposure

Quant Factors	Current	Hist %Rank (1=Top)			
	%Rank	6M	1Y	3Y	5Y
Value	78	81	86	76	94
Growth	49	56	53	53	
Momentum	8	10	32	90	81
Quality	60	54	58	54	85
Low Vol	87	84	82	65	13

Sources for: Style Exposure – J.P. Morgan Global Markets Strategy; all other tables are company data and J.P. Morgan estimates.

See page 42 for analyst certification and important disclosures.

Price Performance



	YTD	1m	3m	12m
Abs	234.6%	-4.2%	23.2%	386.9%
Rel	215.1%	-4.9%	13.7%	355.1%

Company Data

Shares O/S (mn)	116
52-week range (\$)	34.39-5.77
Market cap (\$ mn)	3,444.33
Exchange rate	1.00
Free float (%)	82.6%
3M ADV (mn)	4.22
3M ADV (\$ mn)	113.8
Volatility (90 Day)	110
Index	RUSSELL 2000
BBG ANR (Buy Hold Sell)	11 1 0

Key Metrics (FYE Dec)

\$ in millions	FY25A	FY26E	FY27E
Financial Estimates			
Revenue	62	0	0
Adj. EBITDA	(109)	(206)	(229)
Adj. EBIT	(111)	(208)	(229)
Adj. net income	(102)	(198)	(218)
Adj. EPS	(0.88)	(1.30)	(1.24)
BBG EPS	(0.89)	(1.29)	(1.55)
Cashflow from operations	(139)	(141)	(164)
FCFF	(146)	(152)	(174)
Margins and Growth			
Revenue Growth Y/Y (%)	48.3%	(100.0%)	-
EBITDA margin	(174.7%)	-	-
EBITDA Growth Y/Y (%)	(23.8%)	88.8%	11.2%
EBIT margin	(178.4%)	-	-
Net margin	(163.0%)	-	-
Adj. EPS growth	(26.5%)	48.0%	(4.1%)
Ratios			
Adj. tax rate	(0.0%)	(0.0%)	0.0%
Interest cover	19.4	21.2	23.6
Net debt/Equity	-	-	-
Net debt/EBITDA	-	-	-
ROCE	(40.8%)	(37.5%)	(33.7%)
ROE	(37.3%)	(35.6%)	(32.1%)
Valuation			
FCFF yield	(4.2%)	(3.4%)	(3.4%)
Dividend yield	-	-	-
EV/Revenue	56.0	-	-
EV/EBITDA	NM	NM	NM
Adj. P/E	NM	NM	NM

Summary Investment Thesis and Valuation

Investment Thesis

We have an Overweight rating on TNGX shares. We see TNGX set up well following the strong combinatorial data for its lead PRMT5 inhibitor vopimetostat with RAS inhibitors in 2L+ PDAC which supports its planned Ph 3 clinical trial plans in 1L PDAC. While data is early, the unprecedented efficacy in such a difficult indication supports its penetration into the large portion of PDAC patients with MTAP-deletion. Beyond vopimetostat, Tango's CNS-penetrant PRMT5 inhibitor, TNG456, is currently in Ph 1/2 for the very difficult to treat glioblastoma and provides a meaningful virtual "call option" that we think is not reflected in shares today.

Valuation

Our Dec 2026 price target of \$46 is based on our DCF SOTP calculation. We assume stand-alone contributions from vopimetostat in 1L PDAC and 2L NSCLC as well as TNG456 contributions in GBM. Our \$46 price target consists of \$39/share for vopimetostat in 1L PDAC, ~\$1/share for vopimetostat in 2L NSCLC, \$0.4/share for TNG456 in GBM and the remainder for corporate expenses/pipeline and cash.

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Investment Thesis

Tango Therapeutics (TNGX) Overweight

RAS Inhibitor Combinations Have the Potential to Continue the PDAC Treatment Revolution

While Tango's vopimetostat has shown promise as a monotherapy for PDAC, and Tango planned on initiating a Ph 3, 2L PDAC trial, things in the treatment landscape have changed with RVMD's daraxonrasib impressive showing with imminent approval expected. However, Tango has been able to take advantage of this with their ongoing daraxonrasib combo trial as well as RVMD's G12D KRASi zoldonrasib. Data presented last month was our first look at what is possible when daraxonrasib is combined with a non-overlapping and proven MoA such as PRMT5 inhibition in MTAP-deleted tumors.

The first look at 2L+ PDAC data for the vopimetostat + dara combination has shown a 92% ORR with all but one patient still on drug and a 6-month PFS rate of 90%, a number that is so far ahead of what we've seen in this difficult indication that Tango now plans on starting a pivotal trial in this setting ASAP. As a reference, daraxonrasib monotherapy in 2L PDAC has shown an ORR of 30%-35%, which is also considered extremely high compared to the meager historical response rates seen in PDAC with SoC chemotherapy.

Competition in the monotherapy setting for MTAP-deleted tumors is tough, however, Tango has an opportunity to differentiate itself as the first to show such strong combination data. We expect competitor BMY to come out with data in this combo setting as well as it also has a collaboration with RVMD. However, Tango is also hedging its bets in this setting with a recently announced collaboration with ERAS.

Figure 1: Efficacy comparison in 2L PDAC

Study	Regimen	Line	n	ORR	DCR	mPFS	6-mo PFS	mOS	6-mo OS	mDOR
TNG462-C102	Daraxonrasib 100mg + 200mg vopimetostat	2/3L	9	89%	100%		90%			
	Daraxonrasib 100mg + 250mg vopimetostat	3	3	100%	100%					
Pivotal RASolute 302	Daraxonrasib 300mg G12X		228	33.2%	82%	7.3	59%	13.2	78%	
	Daraxonrasib 300mg ITT	2L	248	31.6%	80%	7.2	56%	13.2	78%	-
	Chemotherapy G12X		231	11.8%	50%	3.5	32%	6.6	54%	-
	Chemotherapy ITT		252	11.2%	50%	3.6	33%	6.7	55%	-
RMC-6236-001	Daraxonrasib 300mg G12X	2L	26	35.0%	92%	8.5	78%	13.1	100%	8.2
	Daraxonrasib 300mg mutRAS		38	29.0%	95%	8.1	66%	15.6	97%	8.2
	Daraxonrasib 300mg G12X	3L+	38	21.0%	87%	4.3	41%	8.6	71%	3.5
	Daraxonrasib 300mg mutRAS		45	20.0%	84%	4.3	41%	9	71%	3.3
RMC-9805-001	Zoldonrasib 1200mg + Daraxonrasib 300mg	2L	30	50.0%	97%	9.6	71%	NE	89%	
	Zoldonrasib 1200mg + Daraxonrasib 300mg	3L+	30	47.0%	90%	7.6	59%	10.5	82%	
AURORAS-1 (US) / JYP0015M101 (CN)	ERAS-0015 24-32mg G12X	2L	12	42.0%	-	-	-	-	-	-
	ERAS-0015 32mg G12X	3L+	16	25.0%	-	-	-	-	-	-
NAPOLI 1	5-FU+LV+Na-IRI	2L+	7	57.0%	100%	-	-	-	-	-
		2L+	117	17.1%	50%	3.1	28%	6.2	53%	-
SWOG S1513	FOLFIRI	2L	58	10.0%	54%	2.9	28%	6.5	55%	3.4
SWOG S1115	FOLFOX	2L	62	7.0%	32%	2	27%	6.7	64%	-
SEQUOIA	FOLFOX	2L	284	5.6%	37%	2.1	19%	6.3	51%	5.2
QUILT-3.010	Gemcitabine + nab-paclitaxel	2L	40	2.9%	24%	2.7	24%	6.6	53%	-
Trybeca-1	Gemcitabine + nab-paclitaxel	2L	148	12.5%	49%	3.5	29%	6.9	58%	5.7
GEMPAX	Gemcitabine + paclitaxel	2L	140	17.1%	53%	3.1	29%	6.4	52%	-

Source: Company reports and J.P. Morgan Research.

Vopimetostat is Part of a Next Generation of PRMT5 Inhibitors Following Many Years of Development and Failure

Vopimetostat was developed as a second generation PRMT5i by Tango, following the discontinuation of TNG908 which was itself part of a new wave of MTA-cooperative

PRMT5is. Earlier PRMT5is worked through less selective mechanisms of action, such as substrate competition, which would inhibit PRMT5 but would not differentiate between healthy and tumor cells, creating a very limited therapeutic window. However, since MTAP-deleted tumors represent such a meaningful portion of all tumors (10%-15%), work continued to develop better, more selective, PRMT5 inhibitors and Tango's drugs are a result of this decades-long effort.

The ability of vopimetostat, TNG456 and other assets to bind and inhibit PRMT5 in the presence of high levels of MTA, which is a direct result of MTAP deletion, allows to differentiate between healthy and tumor cells, widely expanding the therapeutic window with a tolerable safety profile that opens up combination options, which has now become Tango's modus operandi.

Combos and More Combos! Brain-Penetrant TNG456 Proof-of-Concept Coming This Year and a CDK4/6i Combo Soon After

Beyond the combination opportunity with RAS inhibitors in PDAC and NSCLC, Tango also has an ongoing Ph 1/2 trial in glioblastoma with brain-penetrant TNG456 and monotherapy data expected this year. Preclinical data has shown a lot of promise and the opportunity in this indication is meaningful with up to 45% of cases originating from MTAP-deleted tumors. Tango has a collaboration agreement with Eli Lilly and if monotherapy data shows any promise (in an indication with very few treatment options), Tango has said they will initiate the second portion of the study which will combine TNG456 with abemaciclib, LLY's CDK4/6i. While it remains to be seen if this combination will play out in the clinic, the fact that MTAP deletion almost exclusively occurs in the setting of CDKN2A/B deletion, rendering cells sensitive to CDK4/6 inhibition, opens up another interesting combination approach for non-overlapping MOAs in another setting that is ripe for new options.

Focusing on PRMT5 and Combinations: Tango is Hyper-focused with a Leaner Pipeline

Until earlier this year, Tango still had assets in its pipeline with targets beyond PRMT5, such as HBS1L (TNG961) and CoREST (TNG260), however, it recently deprioritized these assets and streamlined the pipeline to focus solely on the PRMT5i opportunity with vopimetostat and TNG456. This follows the promising monotherapy and combination data seen for vopimetostat and the large opportunities they present. Both TNG260 and TNG961 have been deprioritized and we see this as a smart move, allowing the company to execute on the combo opportunities with its PRMT5is by expanding clinical trials to multiple combinations (RAS, CDK4/6 and potentially MAT2A), collaborations (RVMD, ERAS and LLY) and to gain potential differentiation over competition in the space.

Risks to Rating and Price Target

Competitive Risks in the MTAP-Deletion Treatment Landscape

So far, vopimetostat has shown promising efficacy in multiple tumor histologies. However, monotherapy data has not been clearly superior to competition coming from much larger pharma companies. Since the data is still not mature enough to paint a clear differentiated picture, it remains to be seen if there is indeed a benefit to Tango's lead asset over competition using the same mechanism of action.

Lead in Combo Setting Isn't Promised, With Competition Also Teaming Up

Combination data for vopimetostat with daraxonrasib is highly promising, however, BMS is also running a trial in combination with daraxonrasib in similar indications and while Tango has a slight lead, BMS is the larger player, and barring any monotherapy advantage for vopimetostat, with more resources and capacity to run trials in multiple indications it will be hard to differentiate vs navlimetostat.

Cash Needs to Bring Multiple Assets to Center Stage

TNGX will need to raise additional cash to fund ongoing development across its preclinical and clinical assets. There are also non-dilutive avenues for the company, such as company partnerships or discovery collaborations, that could bring in additional funding. TNGX may also consider dilutive resources to fund its ongoing R&D efforts.

Clinical and Regulatory Risks

There are inherent risks associated with clinical trial conduct and regulatory drug approval, which are not atypical for Biotechs at this stage in development. Factors such as trial enrollment, patient screening, and drug availability may delay milestones and our anticipated timeline, driving downside to our model assumptions.

Company Description

Tango Therapeutics (TNGX), is a clinical-stage biotech company headquartered in Boston, Massachusetts, focused on the development of PRMT5 inhibitors for the treatment of MTAP-deleted tumors in NSCLC and PDAC patients. The company's lead asset, vopimetostat, has shown promising data both as a monotherapy and in combination with RAS inhibitors for the treatment of PDAC in Ph 1/2 trials. Tango is in the preparation stages of launching a Ph 3 trial for the combination in 1L PDAC. Additionally, TNG456, Tango's CNS-penetrating PRMT5 inhibitor, is currently in Ph 1/2 trials for the treatment of glioblastoma. Tango is collaborating with Revolution Medicines and Erasca on their pan-RAS inhibitors and with Eli Lilly on their CDK4/6 inhibitor. J.P. Morgan acted as lead bookrunner on Tango's June 2026 follow-on offering.

Company & Portfolio Overview

Clinical-Stage Portfolio Focused on PRMT5 Inhibitors Vopimetostat and TNG456

Tango's clinical stage portfolio has been streamlined this year to focus solely on the PRMT5i opportunity led by vopimetostat and followed by CNS-penetrant TNG456. Vopimetostat is an MTA-cooperative PRMT5 inhibitor (PRMT5i), a next generation class of inhibitors designed to target only cells that have a deletion of the MTAP gene rendering them sensitive to inhibition, a phenomenon termed synthetic lethality.

Figure 2: Overview of TNGX pipeline, as of June 2026

MOLECULE	INDICATION	COMBINATION	CLINICAL PHASE			STATUS
			PRE-CLINICAL	PHASE 1/2	PHASE 3	
Vopimetostat	1L PDAC*	+ daraxonrasib				Study design to be finalized 2H 2026
	2L PDAC	+ daraxonrasib				Expansion of ongoing Phase 1/2 study
	PDAC, Lung	+ daraxonrasib + zoldonrasib				Additional data 2H 2026
	MTAP-del tumors	Monotherapy				Lung data 2H 2026
	PDAC, Lung	+ ERAS-0015				Study start 2H 2026
TNG456	Glioblastoma	Monotherapy				Phase 1/2 data in 2026

*Planned



Source: Company presentation (June 2026).

Vopimetostat is being evaluated as a treatment for MTAP-deleted tumors, a phenotype that is present in 10%-15% of all tumors but is especially common in pancreatic (PDAC), lung (NSCLC) and glioblastoma (GBM) cancers. Vopimetostat has shown monotherapy efficacy in PDAC and other tumor types, with a strong safety profile, opening up the way to evaluation as part of combination therapies with other classes of drugs intended for the same indications, starting with RAS inhibitors such as Revolution Medicine's daraxonrasib and zoldonrasib and more recently Erasca's ERAS-0015. Following strong data with RVMD's daraxonrasib, Tango is planning to initiate a pivotal trial in 1L PDAC.

Moving on to Tango's second asset, also an MTAP-cooperative PRMT5i, TNG456 is designed to have similar potency and selectivity for MTAP-deleted cancer cells as vopimetostat but with much higher BBB crossing properties making it ideal for treating brain cancers such as glioblastoma for which MTAP-deletion prevalence is as high as 45%. TNG456 is currently being evaluated as a monotherapy in a Ph 1/2 trial in MTAP-deleted glioblastoma and if it shows efficacy, cohorts combining TNG456 with Eli Lilly's CDK4/6 inhibitor abemaciclib will be initiated. Tango has guided to an initial data readout in 2H26.

Figure 3: Ongoing TNGX clinical trials

Trial Title	NCT	Status	Pathology	Drugs	Phase	Enrollment	Study Start	Primary Completion
Safety and Tolerability of TNG462 in Patients With MTAP-deleted Solid Tumors	NCT05732831	Recruiting	MTAP-deleted Solid tumors	Vopimetostat Pembrolizumab	1/2	225	5/26/2023	5/1/2026
S095035 as a Single Agent and in Combination in Adult Participants With Advanced or Metastatic Solid Tumors With Deletion of MTAP	NCT06188702	Active, not recruiting	MTAP-deleted Solid tumors	Vopimetostat S095035	1/2	60	4/29/2024	5/1/2027
Safety and Tolerability of TNG456 Alone and in Combination With Abemaciclib in Patients With Solid Tumors With MTAP Loss	NCT06810544	Recruiting	MTAP-deleted Solid tumors	Vopimetostat Abemaciclib	1/2	191	3/24/2025	3/31/2027
Study to Evaluate the Safety, Tolerability & Efficacy of TNG462 in Combination in PDAC & NSCLC Patients	NCT06922591	Recruiting	MTAP-deleted PDAC & NSCLC	Vopimetostat Daraxonrasib Zoldonrasib	1/2	183	5/31/2025	6/30/2027

Source: clinicaltrials.gov

Figure 4: Potential near-term catalysts for TNGX

Program	Indications	Near-term Catalyst	Expected Timing	Significance
Vopimetostat	1L PDAC	Finalize Ph 3 combo RCT design	2H26	+
Vopimetostat	2L+ NSCLC	Vopimetostat NSCLC monotherapy data	2H26	++
TNG456	Glioblastoma	Initial GBM data	2H26	++
Vopimetostat	2/3L PDAC	Present 2/3L PDAC vopimetostat + RAS(ON) data at a scientific conference	2H26	++
Vopimetostat	PDAC/NSCLC	Initiate Ph 1/2 vopimetostat + ERAS-0015 combo study	2H26	+
Vopimetostat	1L PDAC	Initiate Vopimetostat + Daraxonrasib Ph 3 trial	2027E	++

Source: J.P. Morgan estimates, Company data.

Snapshot of Tango Therapeutics

Figure 5: Company overview and potential catalysts

Company Overview					
Company	Tango Therapeutics				
Ticker	NASDAQ: TNGX				
Location(s)	Boston, Massachusetts				
Key Technology Platforms	PRMT5 inhibitors				
Theme	Oncology				
Key Driver(s) of Valuation	Vopimetostat combination data with RAS inhibitors				
Next key catalyst to watch	TNG456 glioblastoma initial data				
Our Thesis	(1) RASi Combos Have the Potential to Continue the PDAC Treatment Revolution (2) Vopimetostat is Part of the Next Generation of PRMT5is (3) Doubling Down on PRMT5 Combos: Tango is Hyper-focused with a Leaner Pipeline (4) Brain-Penetrant TNG456 Proof-of-concept Coming This Year and a CDK4/6i Combo Soon After				

Catalyst Overview					
Catalysts	Asset	Target Indication(s)	Timing	Importance	
Finalize Ph 3 combo RCT design	Vopimetostat	1L PDAC	2H26	+	
Vopimetostat NSCLC monotherapy data	Vopimetostat	2L+ NSCLC	2H26	++	
Initial GBM data	TNG456	Glioblastoma	2H26	++	
Present 2/3L PDAC vopimetostat + RAS(ON) data at a scientific conference	Vopimetostat	2/3L PDAC	2H26	++	
Initiate Ph 1/2 vopimetostat + ERAS-0015 combo study	Vopimetostat	PDAC/NSCLC	2H26	+	
Initiate Vopimetostat + Daraxonrasib Ph 3 trial	Vopimetostat	1L PDAC	2027E	++	

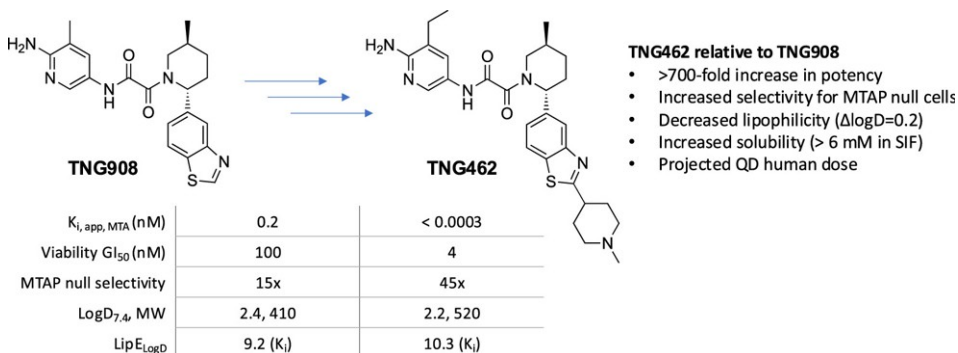
Valuation Overview					
Key Model Assumptions					
Vopimetostat in 1L PDAC					
WACC			10%		
Peak Penetration			60%		
Pricing in US	\$	30,000			
GTN			15%		
Annual Price Increase (US)			4%		
Peak Sales (\$mn) - Risk-adjusted	\$	3,485			
Key Model Assumptions					
Vopimetostat in NSCLC					
WACC			11%		
Peak Penetration			15%		
Pricing in US	\$	30,000			
GTN			15%		
Annual Price Increase (US)			4%		
Peak Sales (\$mn) - Risk-adjusted	\$	150			
Key Model Assumptions					
TNG456 in GBM					
WACC			11%		
Peak Penetration			30%		
Pricing in US	\$	30,000			
GTN			3%		
Annual Price Increase (US)					
Peak Sales (\$mn) - Risk-adjusted	\$	96			

Source: Company reports and J.P. Morgan estimates.

Vopimetostat and TNG456 - Next Generation MTA-Cooperative PRMT5 Inhibitors

Tango's MTA-cooperative PRMT5i program was built around the synthetic-lethality relationship, progressing from the earlier stage TNG908 to Vopimetostat (formerly TNG462) and CNS-penetrant TNG456. TNG908 was developed as a brain-penetrant MTA-cooperative PRMT5 inhibitor for MTAP-deleted cancers, including CNS tumors. Vopimetostat and TNG456 were designed as next-generation PRMT5is with enhanced potency, selectivity, and PK properties to improve target coverage and enable broader, deeper antitumor activity in MTAP-deleted solid tumors. While in preclinical work, TNG908 showed a 15x selectivity for MTAP deleted cells, vopimetostat showed 45x selectivity, and a more potent growth inhibition. For a while both vopimetostat and '908 were in clinical evaluation, however, in November 2024, following promising activity in NSCLC and PDAC, vopimetostat replaced '908 in the clinic for non-CNS tumors. At the same time, Tango unveiled TNG456, which has shown a 55x selectivity for MTAP deletion in preclinical work with high CNS penetration. In May 2025, the first patient was dosed in TNG456's Ph 1/2 trial in MTAP-deleted tumors with a focus on GBM.

Figure 6: Vopimetostat (TNG462) relative to TNG908



Source: Cottrell, Kevin M., et al. "Discovery of TNG462: a highly potent and selective MTA-cooperative PRMT5 inhibitor to target cancers with MTAP deletion." *Journal of Medicinal Chemistry* 68.5 (2025): 5097-5119.

A Short Primer on MTAP Deletion and PRMT5 As a Target in Cancer

Methylthioadenosine phosphorylase (MTAP) is an enzyme encoded on chromosome 9p21, adjacent to the tumor suppressor gene CDKN2A and because of this genomic proximity, MTAP is almost exclusively co-deleted with CDKN2A in human cancers. MTAP loss is observed in a broad range of solid and hematologic malignancies and is generally estimated to occur in approximately 10%–15% of all human cancers. Most importantly, MTAP deletion occurs in ~22% of PDAC, ~13% of NSCLC and ~45% of GBM.

The Roles of MTAP and PRMT5 in the Cell

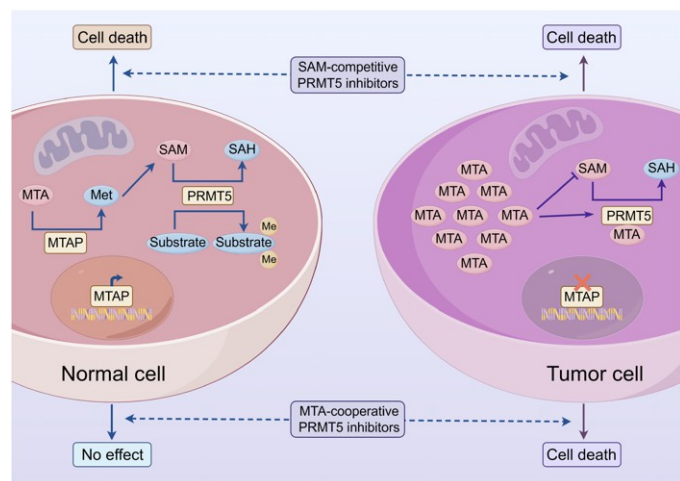
MTAP normally participates in the methionine salvage pathway by metabolizing 5'-methylthioadenosine (MTA) back into methionine. Normal cells do not accumulate high levels of MTA, and PRMT5 can continue using the methyl-donor SAM (S-adenosylmethionine) to perform its normal function. PRMT5 catalyzes symmetric

dimethylation of arginine residues on histones and non-histone proteins and regulates processes central to tumor cell viability, including transcription, chromatin remodeling, RNA splicing, signal transduction, ribosome biology, and cell-cycle progression. When MTAP is deleted, MTA accumulates intracellularly and partially inhibits protein arginine methyltransferase 5 (PRMT5) and so the cell is more reliant on exogenous methionine.

The MTA-Cooperative MoA of Most Current PRMT5is

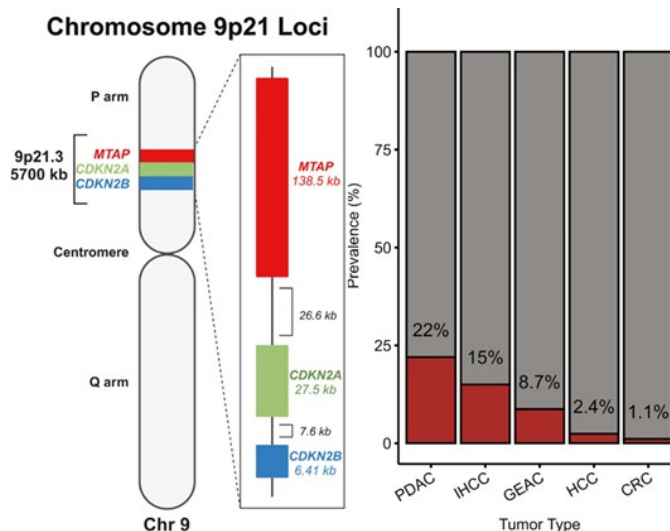
PRMT5 is an essential protein, and so non-selective first-generation PRMT5 inhibitors (PRMT5i) had limiting therapeutic windows with associated hematopoietic toxicity. However, when MTAP is deleted, MTA accumulates, forms a complex with PRMT5 and partially inhibits it. A drug that can bind to PRMT5 only with MTA could lead to cell death, but only in cells with high MTA levels, leading to much better selectivity over healthy cells. In cells with MTAP deletion, PRMT5 is already partially inhibited due to the elevated MTA levels, opening up the possibility of synthetic lethality and sparing of wild-type cells through the development of MTA-cooperative PRMT5 inhibitors, a newer class designed to exploit MTA accumulation. Vopimetostat, and almost all current competitors are MTA-cooperative inhibitors.

Figure 7: Tumors with MTAP deletion are synthetically lethal with PRMT5 inhibition



Source: Tian, Xing, et al. "Methionine restriction in cancer: a dietary insight for therapy." *Frontiers in Nutrition* 13 (2026): 1730639.

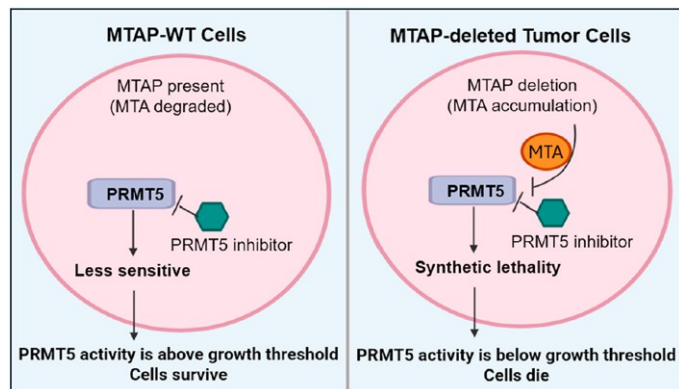
Figure 8: Genomic location of 9p21 and prevalence of MTAP-loss and 9p21 deletion in GI cancers



Source: Ngoi, Natalie YL, et al. "Methylthioadenosine phosphorylase genomic loss in advanced gastrointestinal cancers." *The Oncologist* 29.6 (2024): 493-503.

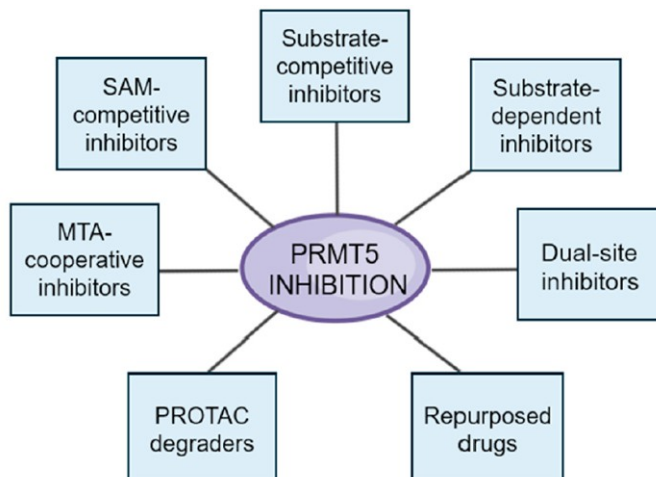
Unlike non-MTA-cooperative or first-generation PRMT5 inhibitors, MTA-cooperative inhibitors preferentially inhibit PRMT5 only when MTA is bound, thereby selectively targeting MTAP-deleted cancer cells, creating a synthetic-lethality strategy.

Figure 9: Mechanism of MTA-cooperative PRMT5 inhibition in MTAP-deleted tumors



Source: Aipourgivi, Faranak, Jing Su, and Tao Lu. "Cracking PRMT5: mechanistic insights, clinical advances, and AI-driven strategies." *Cancer letters* (2025): 21807

Figure 10: Overview of PRMT5 inhibition strategies



Source: Aipourgivi, Faranak, Jing Su, and Tao Lu. "Cracking PRMT5: mechanistic insights, clinical advances, and AI-driven strategies." *Cancer letters* (2025): 21807

Due to it being one of the more common deletions and the central role PRMT5 plays in cancer biology, there have been many attempts to inhibit PRMT5 over the years with different MOAs. However, most have failed to selectively target cancer vs healthy cells and so their development was stopped (for more on other PRMT5i MOAs, see Alipourgivi, et al. "Cracking PRMT5: mechanistic insights, clinical advances, and AI-driven strategies." *Cancer Letters* (2025): 218075).

What are the Highest Value Indications for Vopimetostat?

PDAC - Highest Prevalence and a Real Opportunity to Take Part in a Revolution

Despite recent advances in targeting KRAS for PDAC (see [our work](#) on RVMD), there still exists a significant need to improve outcomes for patients with one of the most deadly cancers. There are an estimated ~66K new pancreatic cancer cases annually in the US, and PDAC comprises at least 90% of pancreatic cancers, followed by neuroendocrine tumors, according the American Cancer Society. Despite the recent Ph 3 success for daraxonrasib, doubling OS in 2L PDAC and as we await survival data from 1L trials, we need to remember that ORR in this patient population was still only ~30% leaving much room for improvement. While PDAC isn't one of the most common cancer types, it is one of the leading causes of cancer death in the U.S. The lethality of PDAC is multifactorial but mostly based on the inability to detect the disease in early stages before it has metastasized. This late stage diagnosis leads to a 5-year survival rate of 3% within those with metastatic disease (and this is after doubling in the past decade, but before RAS inhibitors).

Despite the recent success of daraxonrasib, which is awaiting approval, treatment options for PDAC are fairly limited and not effective: only ~20% of PDAC patients are eligible for surgery due to the advanced stage of the disease at diagnosis. Aside from late diagnosis, there are also risks related to tumor proximity to major vasculature and the lasting impact of removing the pancreas. Even in resection-amenable patients, 75% will develop recurrence within 2 years. Thus, clinical care for metastatic PDAC relies

on systemic chemo. One of the key chemo regimens is FOLFIRINOX (FFX; a combo of 5-fluorouracil (5-FU), leucovorin, irinotecan, and oxaliplatin) with the second being a combo of gemcitabine and nab-paclitaxel (GnP). For progressors, 2L regimens include irinotecan with 5-FU. Following the success of RVMD in the space, there are now dozens of new KRAS-mutant and multi/pan-RAS targeting assets in the clinic with multiple Ph 3 trials assessing their efficacy in 1L PDAC in combination with or without chemotherapy.

Initially, Tango was targeting the MTAP-deleted PDAC population and was planning on starting a Ph 3 trial in 2L MTAP-deleted PDAC, which represents 19%-32% of the PDAC population, according to sources. However, following the recent success of the combo data with multi-RAS inhibitor daraxonrasib, focus is now shifting to starting a Ph 3 pivotal trial in 1L PDAC MTAP-deleted patients in combination with daraxonrasib. In PDAC, there is no need to focus on KRAS mutational status as >90% of cases are KRAS mutated and daraxonrasib is active in WT KRAS as well. Additionally, genomic landscape work has shown a co-mutation rate of ~94% in PDAC. Thus, MTAP-deletion is the only limiting inclusion factor.

NSCLC: The Blueprint for Targeted Therapy

Lung cancer is the second most common type of cancer diagnosed worldwide. The classification of lung cancer has undergone major changes over the years, as evidenced by the increased use of sequencing to classify cancer histology and a shift towards targeted therapy guided by driver mutation characterization. The two main types of lung cancer are non-small cell lung cancer (NSCLC) and small cell lung cancer (SCLC). NSCLC is the most common form, accounting for 80% to 85% of all lung cases, and is further sub-classified into three subtypes: (1) adenocarcinoma, (2) squamous cell carcinoma and (3) large cell carcinoma, which are differentiated based on the tissue of origin.

Adenocarcinoma is the most common form, accounting for ~40% of cases. In adenocarcinoma, the tumor forms in the outer region of the lung where mucus-producing cells reside. In squamous cell carcinoma, which makes up ~25%-30% of cases, the cancer forms in the central part of the airway, in the lining of the bronchi. Squamous cell carcinoma is more common in older adults and smokers. Unlike the other two, large cell carcinoma is much rarer, less centralized, and can be found anywhere in the lungs.

The American Cancer Society estimates the annual incidence of lung cancer cases to be ~230K. The five-year survival rate for all NSCLC is ~28%, but it further worsens as patients enter the advanced stages. Once a tumor is spotted in the lungs, a tissue biopsy is conducted to determine the type of cancer and its stage, while next-generation sequencing is used to determine its genetic signature and the presence of oncogenic driver mutations, which is especially important for Tango's strategy as it will be targeting only tumors with MTAP deletion.

NSCLC treatment is dependent on the mutational characteristics of the tumor. There are multiple types of approved targeted therapies for NSCLC, including inhibitors targeting EGFR, ALK and ROS1 rearrangements, KRAS G12C, MET exon 14, BRAF V600E, RET fusions, HER2 and more being added annually. Absent actionable mutations, 1L treatment is based on the KEYNOTE-189 protocol including Pt-based chemotherapy with immune checkpoint blockade and is dependent on PD-L1 expression levels.

In a large molecular landscape profiling study of NSCLC, MTAP deletion was present in ~13% of cases out of almost 30K samples. Not surprisingly, as they are adjacent on the 9p21 chromosome, the most common co-mutations were CDKN2A and CDKN2B with 98% and 95%, respectively, and TP53 with 63%. Next up were all types of KRAS mutations with 29%. Taken together, co-occurrence of a KRAS mutation with an MTAP-deletion occurs in ~4% of NSCLC cases (Kumar et al.). While seemingly low, with ~190K cases of NSCLC a year in the US, this still amounts to ~25K NSCLC MTAP-deletion cases a year and ~7K with a KRAS co-mutation or ~17K/5K stage III/IV cases.

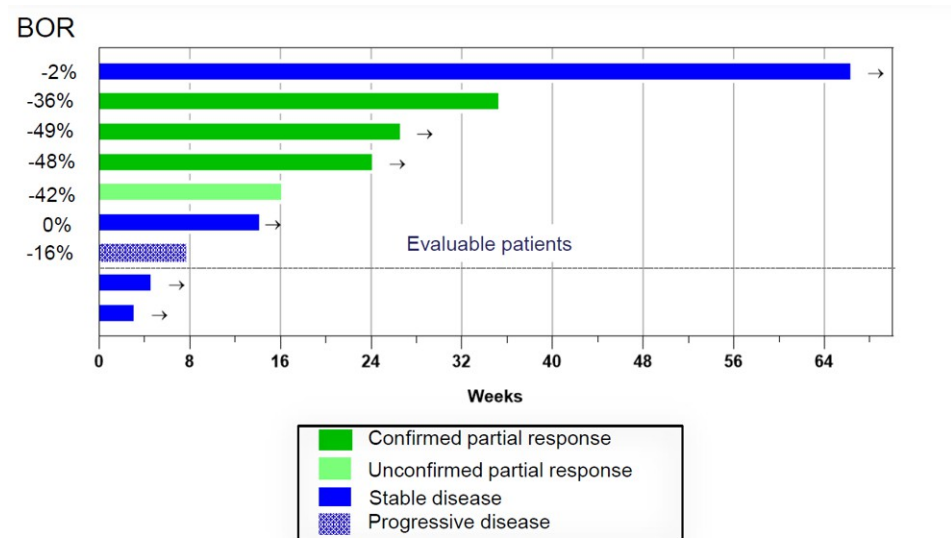
Ashok Kumar, Prashanth, et al. "Genomic landscape of non small cell lung cancer with methylthioadenosine phosphorylase (MTAP) deficiency." *Cancer Medicine* 12.2 (2023): 1157-1166. Hendriks, Lizza EL, et al. "Non-small-cell lung cancer." *Nature Reviews Disease Primers* 10.1 (2024): 71. Ikushima, Hiroaki, et al. "Pan-cancer clinical and molecular landscape of MTAP deletion in nationwide and international comprehensive genomic data." *ESMO open* 10.4 (2025): 104535.

Vopimetostat Clinical Development – Monotherapy

Tango initiated the dose escalation portion of the Ph 1/2 clinical trial for vopimetostat in July 2023 with dose expansion cohorts beginning in June 2024. The trial enrolled only patients with validated MTAP deletion tumors. The dose expansion portion enrolled three cohorts: NSCLC, PDAC and histology-agnostic solid tumors (enriched for cholangiocarcinoma, sarcoma, mesothelioma and bladder cancer).

In November 2024, Tango reported first monotherapy data from the trial with a data cutoff of 10/20/24 and 39 evaluable patients across 13 histologies at 160-300mg QD. At the time, Tango reported RECIST PRs and durable disease control in multiple cancer types. Data showed that the time to response was relatively long with a mTTR of 16 weeks, similar to competitor data. No NSCLC/PDAC data was given at the time but out of 7 cholangiocarcinoma patients, 3 achieved a PR (ORR of 43%) at the time.

Figure 11: Vopimetostat activity in cholangiocarcinoma

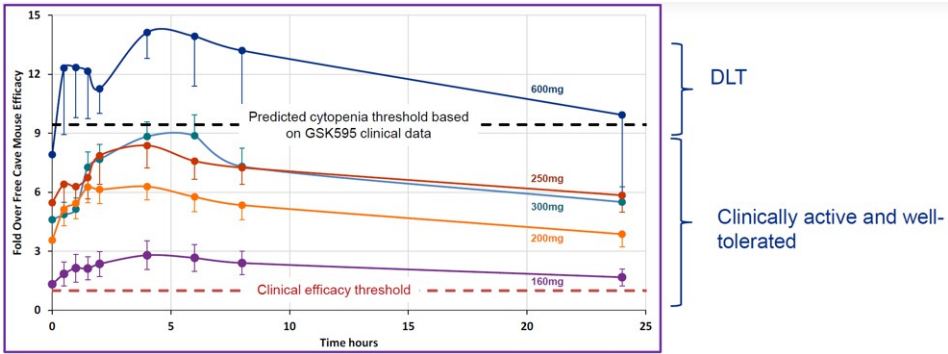


Data cutoff 20 October 2024

Source: Corporate presentation February 2025

Dose expansion from that point focused on 250mg QD and NSCLC/PDAC, and vopimetostat showed a good safety profile at 200mg QD with 0/30 dose reductions, <20% GI tox, and importantly no thrombocytopenia which compared favorably to competition. The MTD was identified as 300mg QD.

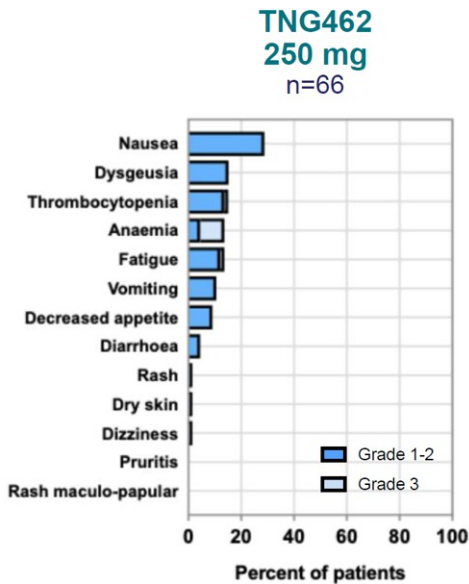
Figure 12: TNG462 exposure at clinically active doses



Source: Corporate presentation May 2025

In July 2025, Tango disclosed more detailed safety data from the relevant 250mg QD cohort for vopimetostat with n=66 showing an 8% dose reduction and slightly higher anemia and thrombocytopenia event rates including Gr3 anemia.

Figure 13: Vopimetostat early safety data



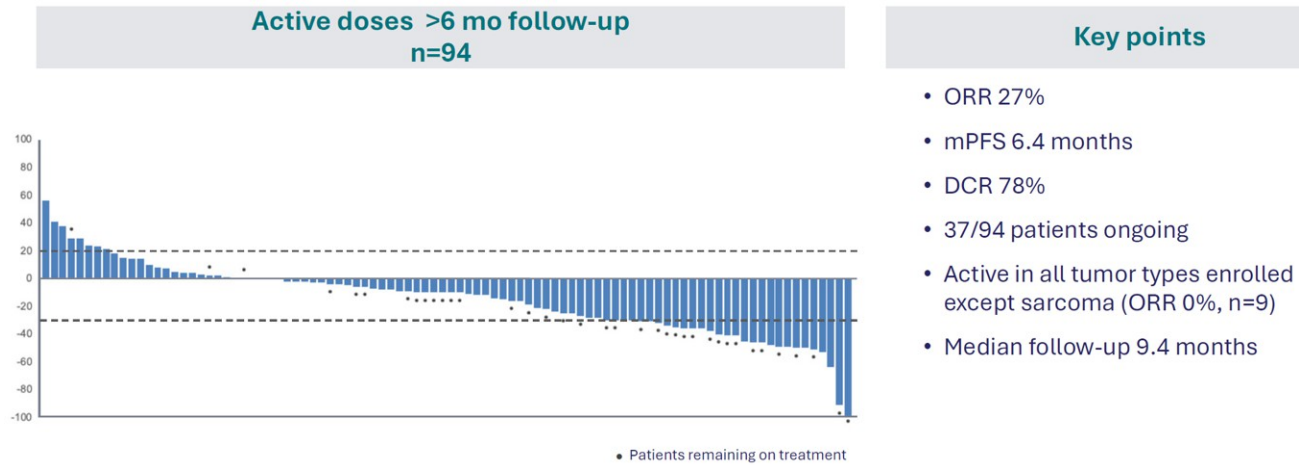
Dose reduction 8%
Median follow-up 4.2 mo
16 June 2025

Source: Company presentation July 2025

Detailed Monotherapy Data

In October 2025, Tango reported detailed monotherapy data for vopimetostat including 94 patients dosed at active doses (≥ 200 mg QD) with >6 months of follow up prior to the cutoff date of 9/1/25. This focus was based on the relatively long time to response seen with competitor PRMT5is and improved ORR from 20% (follow up naïve) to 27% (>6 months follow up) across all tumor types (n=94) with an mPFS of 6.4 months (in first scan eligible patients; n=143). Activity was seen in all enrolled tumor types other than sarcoma where ORR was 0%.

Figure 14: Vopimetostat activity across cancer types



Source: Company presentation November 2025

PDAC

Within the first cohort, there were 64 PDAC patients treated at active doses with n=29 2L and n=34 patients with >6 months of follow up at cutoff.

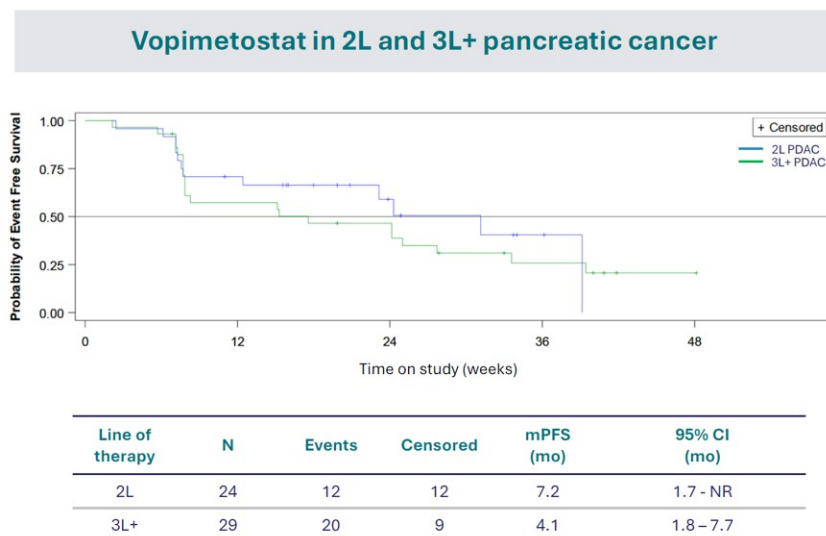
Figure 15: Pancreatic cancer demographics for vopimetostat phase 1/2 study

PDAC Patients	2L	3L+
N	29	34
Median age (years)	64	66
Performance status		
ECOG 0	31%	38%
ECOG 1	69%	62%
Prior therapies		
Prior systemic therapy	100%	100%
Neo-adjuvant therapy	21%	32%
Adjuvant therapy	31%	41%
Advanced/metastatic setting	100%	100%
Prior lines of therapy		
1	100%	NA
2	NA	56%
3	NA	27%
>=4	NA	18%

Source: Company presentation November 2025

In the 29 2L PDAC patients with MTAP deleted tumors, mPFS was 7.2 months and ORR in those with >6 months of follow-up was 25%. In 3L+ patients (n=29), mPFS was 4.1 months. ORR for all PDAC patients was 15%. At that point in time, Tango planned on initiating a pivotal trial in 2L MTAP-deleted PDAC patients in 2026 (these plans later changed following combo data with RVMD's RAS inhibitors).

Figure 16: Vopimetostat monotherapy efficacy in 2L and 3L+ pancreatic cancer



Source: Company presentation November 2025

Summary

Vopimetostat

- 2L mPFS 7.2 mo
- 2L ORR 25%
 - Overall ORR 15%
- Overall DCR 71%
- Median follow-up 7.8 mo

Chemotherapy historical control trials

- 2L mPFS 2-3.5 mo
- 2L ORR ~10%*

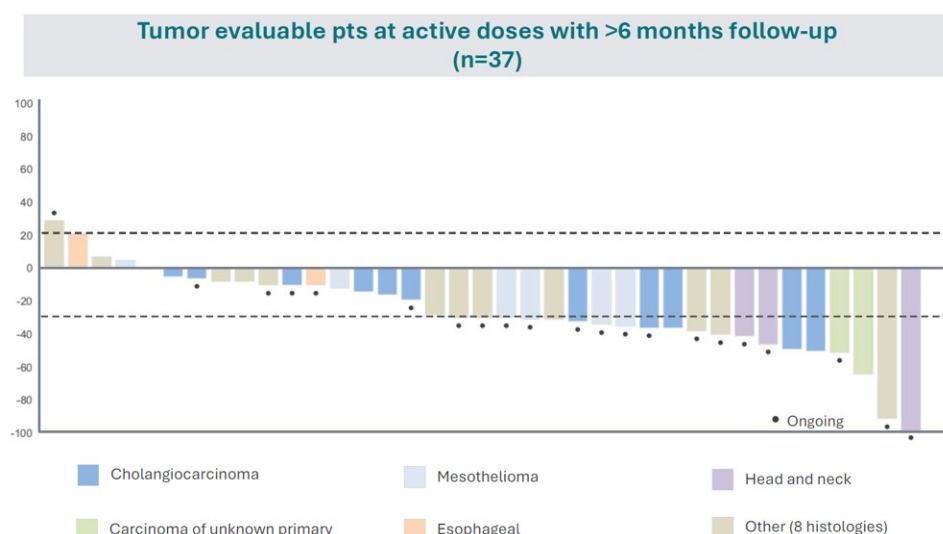
NSCLC

As of the cutoff date of 9/1/25, there were 41 2L+ lung cancer patients enrolled in the trial at active doses with only 12 having >6 months of follow-up. Tango didn't provide material updates at the time and a more thorough disclosure is expected in 2H26. As of June 2026, the lung cohort is fully enrolled.

Histology Selective Cohort

Finally, in the histology-selective cohort, there were 37 evaluable patients with an ORR of 49%, DCR of 89% and mPFS of 9.1 months. This cohort did not include PDAC/ NSCLC patients. In sarcoma, no responses were recorded out of 9 patients and so this indication will not be pursued.

Figure 17: Vopimetostat activity in histology selective cohort



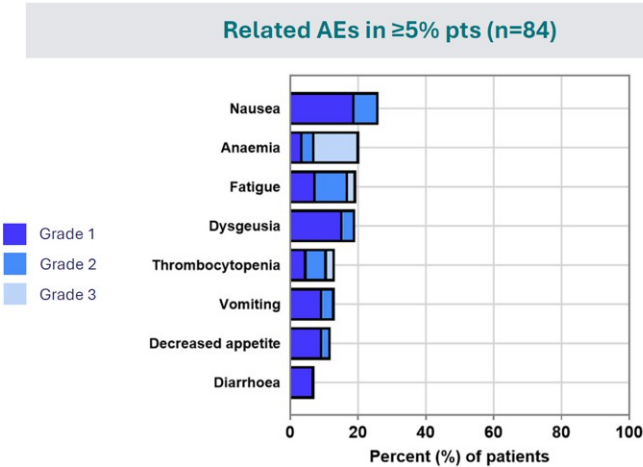
Source: Corporate presentation November 2025

Key points

- ORR 49%
- mPFS 9.1 months
- DCR 89%
- 21/37 patients ongoing
- Median follow-up 9.5 months
- Excludes sarcoma, pancreatic and lung cancer

Consistent with the previous safety data cutoff, now with n=84 and a longer follow-up of 6.1 months, there were still 8% dose reductions and no discontinuations or Gr4+ AEs. The most common TRAEs, mostly Gr1, were nausea (26%), anemia (20%), fatigue (19%), dysgeusia (19%) and thrombocytopenia (13%). Gr3 anemia was observed in 13% of patients.

Figure 18: Vopimetostat 250 mg QD safety profile



Source: Corporate presentation November 2025

The Case for Co-Targeting PRMT5 with Other Targets

MTAP-loss occurs almost exclusively as part of 9p21 chromosomal loss with co-deletions of CDKN2A and CDKN2B. Prior genomic studies, and even commercial panels, have not always tested specifically for MTAP-loss, making it challenging to perform retrospective studies on the clinical outcomes of patients with tumors harboring MTAP-loss. Testing for CDKN2A and CDKN2B is nearly ubiquitous, though, and this finding allows us to extrapolate the presence of MTAP-loss or perform dedicated testing for MTAP-loss in select scenarios.

Although current MTAP-loss clinical trials are investigating MAT2A or PRMT5 inhibitors as monotherapy, combination targeted therapy may ultimately prove more efficacious.

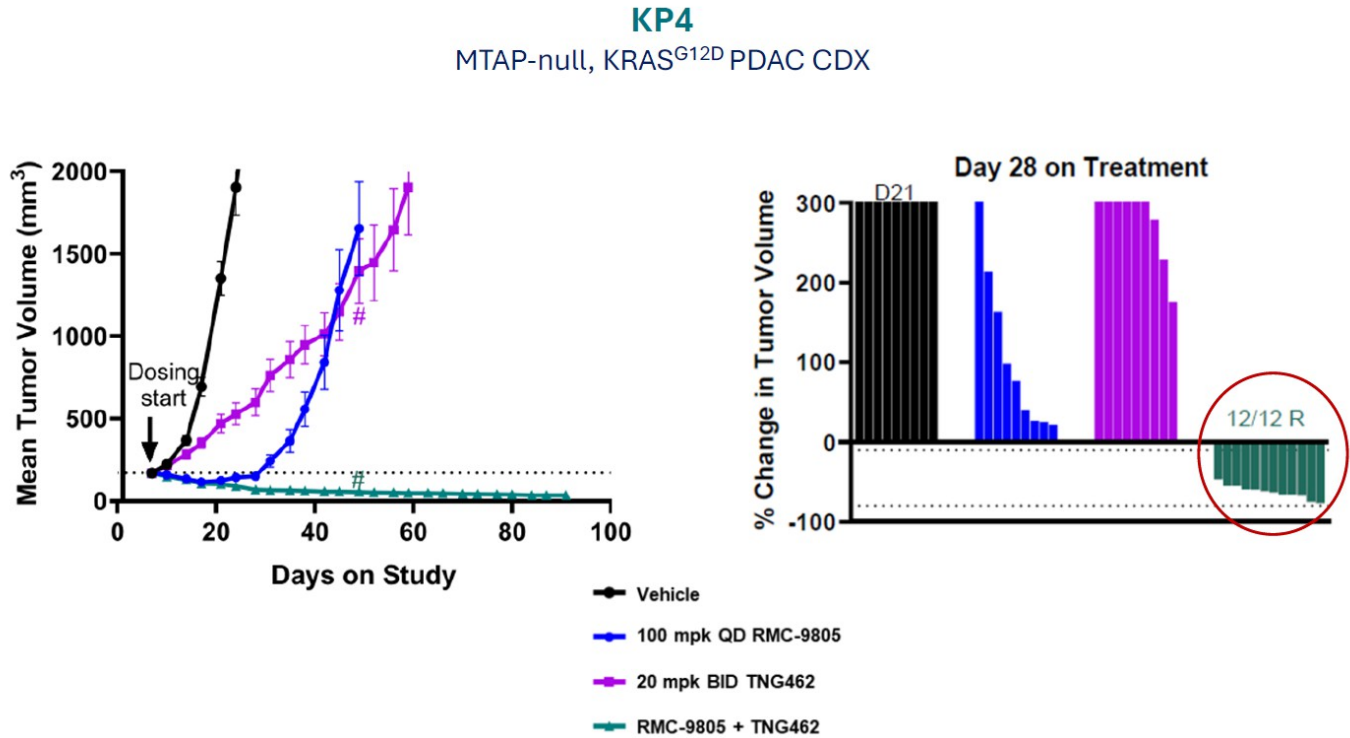
Building on the improved safety profile seen with the MTA-cooperative PRMT5i, combining them with other targeted cancer therapeutics makes sense, especially since MTAP-deleted tumors often harbor additional actionable oncogenic drivers that can be inhibited in parallel, opening up the possibility to improve on the monotherapy data seen with vopimetostat and competitors.

An especially strong rationale exists for combining KRASi with PRMT5i in MTAP deleted PDAC, where >90% of all tumors are KRAS mutation-driven, which is believed to be the founding oncogenic event, while CDKN2A/MTAP deletion is another frequent early alteration. PRMT5 inhibition and KRAS inhibition largely affect distinct signaling networks, but their downstream consequences converge on essential tumor processes such as MYC signaling, cell-cycle control, DNA and RNA biology, translation, interferon signaling, and apoptosis. Because these mechanisms are at least partly nonoverlapping, combinations may deepen responses, delay resistance, and produce more durable tumor regressions than single-agent therapy, while maintaining tolerability if each component has an adequate therapeutic index.

Preclinical studies with PRMT5 inhibitors combined with mutant-selective RAS inhibitors showed greater growth suppression, impaired cell division, tumor regression, and delayed tumor outgrowth compared with either agent alone.

Tango has presented preclinical work supporting the combination of vopimetostat with zoldonrasib (RVMD), showing more durable tumor regression, as well as with the EGFRi osimertinib (Tagrisso).

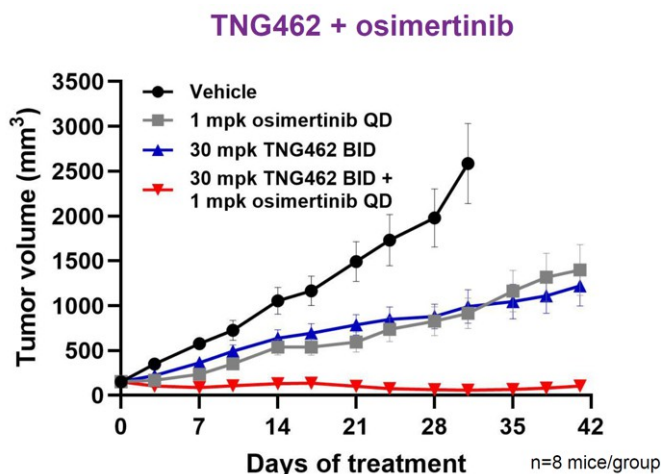
Figure 19: Vopimetostat + RAS inhibition activity in preclinical models



Source: Corporate presentation November 2025

The same principle extends beyond KRAS-driven cancers. In MTAP-deleted tumors with EGFR activation, combining TNG462 with EGFR inhibition, such as osimertinib, is supported by the idea that PRMT5 inhibition targets the MTAP synthetic-lethal state while EGFR blockade suppresses the dominant oncogenic signaling driver. Similarly, because MTAP deletion is usually accompanied by CDKN2A deletion, these cancers may have increased CDK4/6 activity, creating a rationale for combining PRMT5 inhibitors with CDK4/6 inhibitors (CDK4/6i) such as palbociclib or abemaciclib.

Figure 20: Vopimetostat + EGFR inhibition activity in preclinical models



Source: Zhang, Minjie, et al. "TNG462, an MTA-cooperative PRMT5 inhibitor, demonstrates strong efficacy in combination with clinically relevant targeted therapies in MTAP-null preclinical models." *Cancer Research* 85.8_Supplement_1 (2025): 2996-2996

This work prompted Tango to initiate combination trials in collaboration with RVMD (entered November 2024) testing daraxonrasib and zoldonrasib in combo with vopimetostat in PDAC and NSCLC, with the first patients dosed in June 2025. During their 1Q25 update, TNGX announced that the company's combination focus was narrowed to only RAS inhibitor work, starting with RVMD. Later in March 2026, Tango announced a collaboration agreement with ERAS to combine vopimetostat with ERAS-0015, the company's pan-RAS inhibitor. The goal is to establish vopimetostat as the preferred PRMT5i for combination with RAS-targeted therapies.

RVMD also established collaboration agreements with competing PRMT5i navlimetostat (formerly BMS-986504 from BMY, entered February 2026) and the now suspended AMG 193 (AMGN; entered February 2025).

While vopimetostat focus is currently only on RAS combinations, Tango also has a clinical collaboration agreement with LLY to combine TNG456 (CNS-penetrant PRMT5i) with LLY's CDK4/6i abemaciclib to be tested in a Ph 1/2 clinical trial focused on MTAP-deleted GBM. Tango has said that once there is a single-agent PoC with TNG456, they will commence combo treatment in patients.

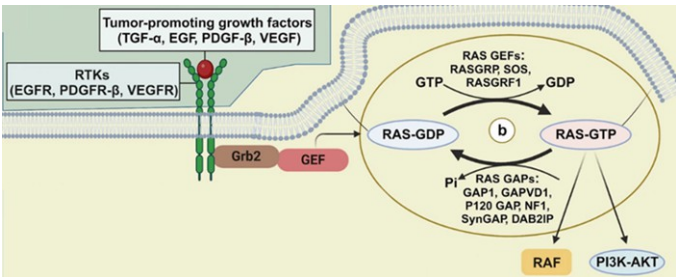
A Quick Primer on RAS

RAS serves as a central node in the RAS/RAF/MAPK pathway, which holds a key role in regulating cell behavior by relaying extracellular signals to the nucleus to control cell growth, proliferation, and differentiation. Dysregulation of the pathway through gain-of-function mutations in the cascade is a frequent driver of cancer formation.

The RAS family of proteins, including KRAS, NRAS and HRAS, are small GTPases that function as switches, alternating between GDP-bound (OFF) and GTP-bound (ON) states. Activation can be triggered by various stimuli. In the normal state, it is recruited to the plasma membrane following stimulation by growth factors, which leads GEF to exchange GDP for GTP turning the switch ON. In the ON state, RAS interacts with a diverse set of effectors, including the RAF proteins and PI3K, both of which are central regulators of cell functions including growth, metabolism, motility, and proliferation

(Figure 7). For more information, see our initiation on RVMD ([here](#)).

Figure 21: The importance of RAS/RAF/MAPK pathway in oncogenesis



Source: Bahar, Md Entaz, Hyun Joon Kim, and Deok Ryong Kim. "Targeting the RAS/RAF/MAPK pathway for cancer therapy: from mechanism to clinical studies." *Signal transduction and targeted therapy* 8.1 (2023): 455.

Figure 22: The frequency of RAS mutations in cancer

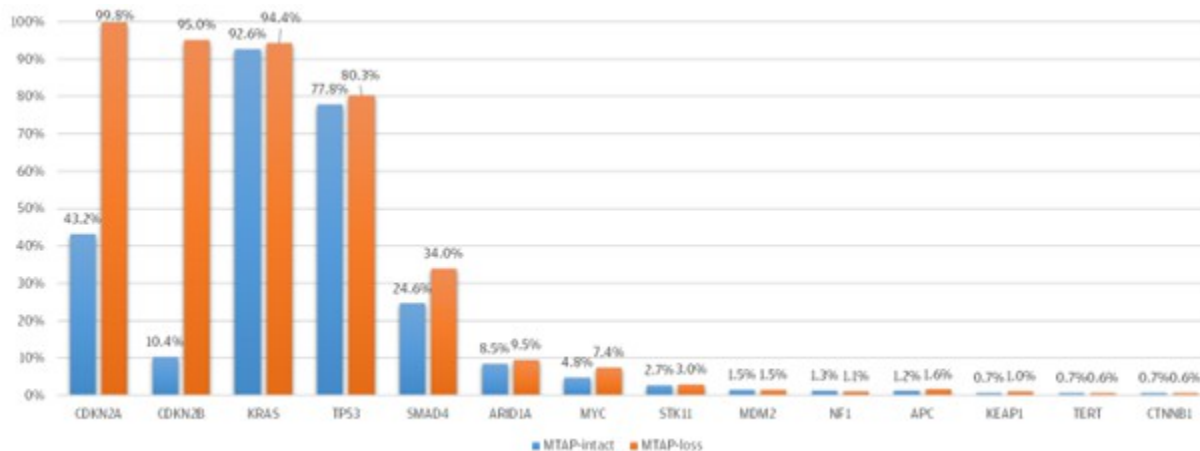
Cancer type	HRAS	KRAS	NRAS
Lung adenocarcinoma	0%	32%	1%
Lung squamous cell carcinoma	1%	4%	1%
Invasive breast cancer	1%	1%	0%
Esophageal cancer	0%	2%	0%
Stomach adenocarcinoma	1%	4%	1%
Small intestine adenocarcinoma	0%	26%	1%
Colon adenocarcinoma	1%	50%	4%
Rectal adenocarcinoma	0%	50%	4%
Liver hepatocellular carcinoma	0%	1%	1%
Cholangiocarcinoma	0%	23%	4%
Gallbladder carcinoma	1%	16%	2%
Pancreatic ductal adenocarcinoma	0%	88%	0%
Clear cell kidney carcinoma	0%	1%	0%
Urothelial bladder cancer	7%	5%	1%

Source: Prior, Ian A., Fiona E. Hood, and James L. Hartley. "The frequency of RAS mutations in cancer." *Cancer Research* 80.14 (2020): 2969-2974

Given their central roles in proliferation and differentiation, it is not surprising that activating mutations in RAS proteins are putative drivers of cancer, with an estimated 30% of all cancer cases driven by mutations in RAS. Oncogenic mutations in RAS are gain-of-function in nature, leading to constitutive activation of signaling in the GTP-bound ON state. Mutations in RAS are usually clustered in codons 12, 13 and 61 (not all mutations are equal in their oncogenicity and impact on survival).

The distribution of RAS mutations among different types of cancers, and among KRAS, HRAS and NRAS, and the type of mutations is variable. Large databases exist with enough inputs to estimate these frequencies. While in PDAC, since most tumors are already RAS mutant (>90% and 40% G12D), the overlap with MTAP deletion is almost perfect (see graph below), in NSCLC, ~30% of cases are RAS mutant, and MTAP deletion is present in ~13% of cases, there is a ~4% co-mutation prevalence which is still meaningful considering the high prevalence of NSCLC (~190K annually in the US). PDAC, with ~66K annual cases in the US, remains the larger target market and the main focus for Tango.

Figure 23: Comparison of differences in genomic alterations between MTAP-loss (right) and MTAP-intact (left) in PDAC tumors.



Source: Ngoi, Natalie YL, et al. "Methylthioadenosine phosphorylase genomic loss in advanced gastrointestinal cancers." The Oncologist 29.6 (2024): 493-503.

Combo Data with RAS Inhibitors

On 6/8/2026, TNGX reported first combo data for vopimetostat with daraxonrasib and zoldonrasib. A total of 59 patients enrolled with previously-treated MTAP-deleted and mutRAS PDAC or NSCLC. All PDAC patients were 2L or 3L in the metastatic setting with >70% having liver mets (in RASolute 302 this was 67%).

Figure 24: Vopimetostat and RASi patient baseline characteristics

PDAC Patients	Dara PDAC	Dara NSCLC	Zoldon PDAC
N	20	5	34
Sex, male (%)	45%	60%	53%
Median age (years)	66	55	66
Performance status			
ECOG 0	60%	40%	26%
ECOG 1	40%	60%	74%
Prior lines of therapy			
0	5%	-	-
1	30%	20%	44%
2	65%	60%	56%
3	-	20%	-
Median pLOT	2	2	2
Liver mets (%)	70%	20%	77%

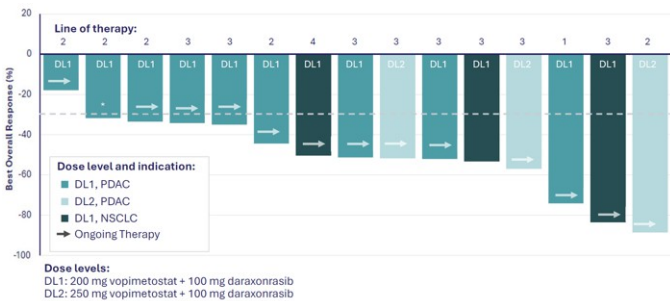
Source: Corporate presentation June 2026

Combo with Daraxonrasib

In the dara portion of the study, patients were treated with 100mg QD daraxonrasib with either 200mg or 250mg of vopimetostat QD, both PO. Tango emphasized the PK/PD modeling work simulated that this would lead to a similar exposure to 300mg QD dara (RASolute 302 dose). A total of 15 patients available for efficacy, which was defined as having ≥14 weeks from first dosing to include at least two CT scans.

The combination of vopimetostat with dara showed very high efficacy with a 92% ORR in the 12 PDAC patients or 93% in the 15 patients inclusive of the 3 NSCLC patients. As a comparison, daraxonrasib 300mg QD monotherapy in RASolute 302, led to an ORR of 29% in mutRAS patients and 35% in G12X patients, while in 3L+ patients this was ~20%. As of the data cutoff, 5/28/2026, 87% of patients remained on treatment, including 2/3 with NSCLC and 11/12 with PDAC. In total, there were 12 confirmed PRs and 2 pending PRs with only one PDAC patient in SD but still on therapy with over 6 months of treatment.

Figure 25: Vopimetostat + Daraxonrasib waterfall plot



Source: Corporate presentation June 2026

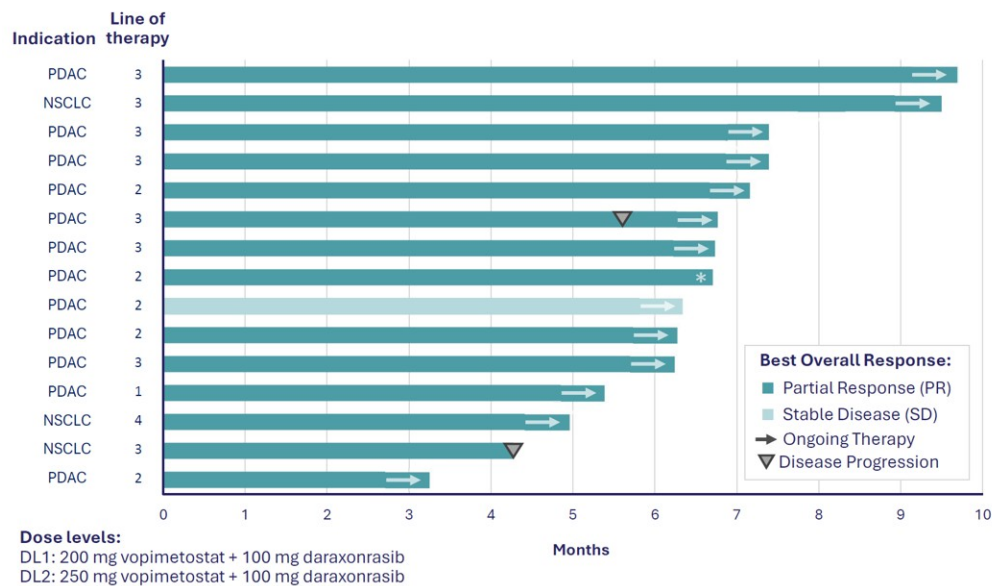
Figure 26: Vopimetostat + Daraxonrasib efficacy summary

	n	ORR	DCR	Ongoing	mFU (months)
PDAC all	12	92%	100%	92%	6.7
PDAC DL1 (200mg vopimetostat + 100mg daraxonrasib)	9	100%	100%	89%	-
PDAC DL1 (250mg vopimetostat + 100mg daraxonrasib)	3	67%	100%	100%	-
PDAC 2L	5	80%	100%	80%	6.3
PDAC 3L	6	100%	100%	100%	7.1
NSCLC DL1	3	100%	100%	67%	5.0
Total	15	93%	100%	87%	6.7

Source: Corporate presentation June 2026

Out of the 15 patients, there were two progressors, one with NSCLC and one with PDAC (still on tx) who both achieved PR initially. Median duration of treatment was 6.7 months (slightly shorter for the 3 NSCLC patients) with a current 6-month PFS rate of 90% in PDAC or 92% in total (~88% in 3L). While still small numbers, in comparison, in RASolute-302, 6-month PFS was ~78% in 2L G12X patients and 66% in 2L mutRAS patients while in 3L this was 41% for both groups. There were 9 other patients who still hadn't reached 14 weeks for efficacy evaluation at that point with no evidence of PD.

Figure 27: Vopimetostat + Daraxonrasib swimmer plot



Source: Corporate presentation June 2026

Key Metrics

- Two disease progressions observed
- Longest treatment ~10 months, ongoing
- Median PFS NR (NE-NE)
- 90% 6-month PFS rate in PDAC (95% CI: 47-99)
- 50% of PDAC patients are 3L
- Patients not yet efficacy evaluable (n=9) are all ongoing with no disease progression

Figure 30: Summary of safety for Vopimetostat + RASi combinations vs. monotherapy

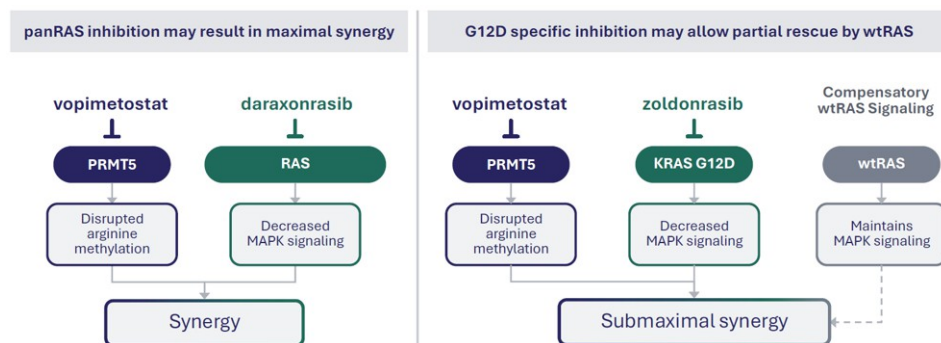
	DL1 PDAC		DL1 NSCLC		DL2 PDAC		Zoldon all combo		Vopi 250mg mono	
	All	Gr3	All	Gr3	All	Gr3	All	Gr3	All	Gr3
n	16		5		4		34		84	
Any event	75%	25%	100%	0%	100%	75%	82%	26%	-	-
Rash*	44%	0%	100%	0%	100%	25%	24%	0%	-	-
Diarrhea	31%	0%	80%	0%	75%	0%	21%	3%	7%	0%
Stomatitis/mucositis	56%	0%	20%	0%	25%	25%	-	-	-	-
Fatigue	13%	0%	20%	0%	75%	25%	21%	0%	17%	2%
Thrombocytopenia	25%	13%	20%	0%	25%	0%	-	-	13%	2%
Nausea	13%	0%	20%	0%	25%	0%	35%	0%	26%	7%
Peripheral edema	13%	0%	0%	0%	50%	0%	-	-	-	-
Anemia	-	-	-	-	-	-	24%	18%	20%	10%
Vomiting	-	-	-	-	-	-	35%	0%	13%	4%
Dysgeusia	-	-	-	-	-	-	18%	0%	19%	4%
Decreased Appetite	-	-	-	-	-	-	-	-	12%	3%

Source: Company reports.

Why is the data with Daraxonrasib better than Zoldonrasib

One question that came up from the data was about the worse performance for the zoldonrasib combo, considering it has generally shown similar efficacy in the G12D population as a monotherapy. While daraxonrasib inhibits mutant RAS in many subtypes as well as the WT protein, zoldonrasib only targets the G12D allele (which was a prerequisite to enter the trial). The goal of the combo is to target two separate pathways that cancer cells depend on. Zoldonrasib has very little activity against WT RAS and preclinical data supports the idea that targeting the mutant allele can lead to compensatory upregulation of WT or other mutant forms of RAS. This could potentially explain the higher efficacy seen with dara in the combo. Another explanation by management was that the ECOG status for the G12D cohort was worse than that for the dara population (see table above).

Figure 31: Possible reasoning behind superior Daraxonrasib combination efficacy



Source: Corporate presentation June 2026

Strategy and next steps

Based on the combo data presented, Tango disclosed its intention to prioritize the vopimetostat and dara combo in the 1L PDAC setting, where it can be developed as a potentially chemo-sparing option with much improved tolerability. Tango now plans to initiate a Ph 3 placebo-controlled RCT in 1L MTAP-deleted PDAC with DL1 of 200mg vopimetostat and 100mg daraxonrasib QD PO. Beyond the planned Ph 3, Tango is expanding the ongoing Ph 1/2 trial to evaluate additional options for a 2L registrational trial. Management has said that they intend to discuss these data with the FDA and work

to confirm the development plans alongside accruing more data.

Competitor PRMT5i Data

There is now a decent list of competitors in the PRMT5i space with at least 15 MTA-cooperative PRMT5i, a handful of MAT2A inhibitors, one substrate-competitive inhibitor and others for whom we couldn't find an MoA. Over time, the competitive risk for Tango has revolved around its comparison to Bristol's navlimetostat (previously GSK-986504 or MRTX-1719) and Amgen's anvmetostat, now discontinued (as of 4/30/26) with Astra's AZD3470 a more recent entrant. Only navlimetostat is currently in Ph 2/3 trials.

Figure 32: PRMT5i competitive landscape

Drug Name	Type	Lead Company	Ticker	Indication Name	Current Phase
Vopimetostat	MTA- cooperative inhibitor	Tango	TNGX	NSCLC/PDAC	III
TNG456	MTA- cooperative inhibitor	Tango	TNGX	Glioblastoma	I/II
Navlimetostat	MTA- cooperative inhibitor	Bristol-Myers Squibb	BMJ	NSCLC/PDAC	II/III
CTS3497	MTA- cooperative inhibitor	CytosinLab		Solid Tumors	I
PEP-08	MTA- cooperative inhibitor	PharmaEngine	4162	Solid Tumors	I
BGB-58067	MTA- cooperative inhibitor	BeOne	ONC	Solid Tumors	I
BAY 3713372	MTA- cooperative inhibitor	Bayer	BAYN	Solid Tumors	I/II
AZD3470	MTA- cooperative inhibitor	AstraZeneca	AZN	Hodgkin's Lymphoma	I/II
ABSK131	MTA- cooperative inhibitor	Abbisko		Solid Tumors	I
GH56	MTA- cooperative inhibitor	Genhouse		Cancer	I
IDE892	MTA- cooperative inhibitor	IDEAYA	IDYA	NSCLC/PDAC	I/II
GS-2426	MTA- cooperative inhibitor	Gilead	GILD	Solid Tumors	I
CS08399	MTA- cooperative inhibitor	Chipscreen		Solid Tumors and Lymphoma	I
HSK41959	MTA- cooperative inhibitor	Haisco		Solid Tumors	I
JB1-778	Substrate competitive inhibitor	Jubilant		Solid Tumors	I
ISM3412	MAT2A Inhibitor	InSilico		Solid Tumors	I/II
SY-9453	MAT2A Inhibitor	Shouyao		Solid Tumors	I
IDE397	MAT2A Inhibitor	IDEAYA	IDYA	Solid Tumors	I
S095035	MAT2A Inhibitor	Servier		Solid Tumors	I/II
GH31	MAT2A Inhibitor	Genhouse		Solid Tumors	I
HS-10587		Hengrui		Solid Tumors	I
GS-5319		Gilead	GILD	Solid Tumors	I
DC50292A		Heronova		Solid Tumors	I
HRS-6719		Shandong		Solid Tumors	I/II

Source: J.P. Morgan Research

Navlimetostat

Navlimetostat is being studied in MountainTAP-29 and MountainTAP-30, both Ph 2/3 trials. MountainTAP-29, in NSCLCm is studying navli in combination with pembrolizumab + chemo in two doses for Ph 2 and then a dose will be selected for Ph 3. In both, the control arm is pembrolizumab + chemo with a projected readout in 2031. MountainTAP-30 has a similar design but in 1L PDAC and navli is being combined with gemcitabine + nab-paclitaxel (GnP) with a projected readout in 2029.

Beyond the Ph 2/3 trials, navli is also being studied in multiple combinations and tumor histologies in a the Ph 2 MountainTAP-5 including combos with: pumitamidg (anti PD-L1/VEGF bispecific) and chemo in NSCLC, daraxonrasib + GnP in PDAC, temozolomide and radiotherapy in GBM and Opdualag (anti-PD-1 + anti-LAG-3) in melanoma.

Overall, when comparing the monotherapy data between navli and vopimetostat in PDAC, it looks like vopimetostat has the slight ORR advantage, however, this is when we look only at the 2L PDAC patients without the 3L+ patients which brings the ORR down to 15% on par with navli. However, Tango currently has the advantage in the RASi combo data, which Bristol has only started studying later on.

Figure 33: Comparison of PRMT5i efficacy

	Navlimetostat								Vopimetostat					
	AACR 2024		ASCO 2025		WCLC 2025	ESMO TAT 2026		ASCO 2026	Monotherapy		Dara combo		Zoldonrasib Combo	
Histology	NSCLC	PDAC	NSCLC	PDAC	NSCLC	PDAC	NSCLC	NSCLC	2L PDAC	Mix	PDAC	NSCLC	PDAC	
Cutoff date	9/19/2024		2/7/2025		5/7/2025		9/22/2025		9/22/2025		9/1/2025		5/28/2026	
mFU (mos)	7.6		11.1 11.1		13.4		15.4 6.6		15.9		7.8 9.5		5.6	
n (efficacy)	26	33	33	35	35		40	11	35		24	37	27	
Combo	-		-		-		GnP		-		-		-	
Dose					50-800mg						Dara 100mg		Zoldon 600/1200mg	
											200-250mg			
ORR (%)	31%	15%	30%	17%	34%		18%	46%	34%	25%	49%	92%	100%	52%
DCR (%)	81%	70%	79%	69%	80%		72%	73%	80%	71%	89%	100%	100%	96%
mDOR (mos)	10.5	NR	10.5	NR	10.5		NR	NR	9.1	-	-	-	-	-
mPFS (mos)	-	-	-	-	-		-	-	-	7.2	9.1	-	-	-

Source: J.P. Morgan Research, Company data.

While it is hard to compare head to head here as data presentation and patient baselines make it difficult, it appears that safety overall is slightly better for navli than for vopimetostat, but not by meaningful levels. The major caveat here is that the navli Gr3 data was presented for all doses from 50-800mg, and so might be skewed slightly lower.

Figure 34: Comparison of PRMT5i safety

	Navli all 200-600mg		Vopi 250mg mono	
	All	Gr3	All	Gr3
n	112		84	
Any event	78%	12%	-	-
Rash	7%	0%	-	-
Diarrhea	20%	1%	7%	0%
Thrombocytopenia	9%	0%	13%	2%
Nausea	36%	0%	26%	7%
Peripheral edema	4%	0%	-	-
Anemia	10%	2%	20%	10%
Vomiting	26%	0%	13%	4%
Dysgeusia	8%	0%	19%	4%
Decreased Appetite	13%	0%	12%	3%

Source: Navlimetostat AACR 2024 presentation and Tango corporate presentation June 2026

TNG456 - Tango's CNS-Penetrant PRMT5i

Why Do We Need a CNS-Penetrant PRMT5i?

Following their first generation TNG908, Tango developed two additional assets with much better potency – vopimetostat and TNG456. While vopimetostat is slightly more potent than TNG456, it did not achieve CNS exposure, TNG908 did, but was much less potent and less selective for MTAP deletion than either of the newer assets.

Figure 35: Comparison of Tango's PRMT5i properties

	Potency	MTAP selectivity	Clinical CNS exposure
TNG456	20 nM	55X	0.5-1X plasma (predicted)
TNG462	4 nM	45X	-
TNG908	110 nM	15X	0.3X plasma (observed)

Source: Company reports.

Preclinical work by Tango showed that to achieve maximum monotherapy efficacy could require exposure levels so high they could also inhibit WT PRMT5 limiting the therapeutic window. As glioblastoma (GBM) is one of the cancer types with high MTAP deletion prevalence, it made sense to develop a drug with adequate brain exposure. To achieve proper brain penetration, the ratio of unbound brain-to-plasma ($K_{p,uu}$) is a key parameter to predict the levels of peripheral exposure needed to achieve target brain exposures and would ideally be close to 1 to not affect the therapeutic window. For TNG908 this ratio was predicted to be 0.9 but the 15x selectivity vs WT was assumed to not be high enough and so it wouldn't be possible to achieve adequate CNS concentrations. Based on this, Tango discontinued the TNG908 trial in November 2024.

Tango aimed to generate a molecule that can achieve brain exposure higher than the MTAP-deleted GI_{50} but lower than the WT GI_{50} in the periphery to reduce on-target PRMT5 tox in non-tumor cells.

The preclinical data described below show that TNG456 has high potential to be brain penetrant in humans based on preclinical in vitro and in vivo data. Based on this, in May 2025, Tango started enrolling patients in the Ph 1/2 clinical trial for TNG456 as a monotherapy. The trial is currently in the dose escalation phase and assuming favorable monotherapy data, a combination with abemaciclib (Eli Lilly's CDK4/6i aka Verzenio) will be initiated. An update has been guided to 2H26.

Glioblastoma Background

Glioblastoma, considered the most aggressive primary brain tumor in adults, has an incidence of ~12K cases a year (3.2/100K) in the US or ~15% of all primary brain and CNS tumors and 45% of malignant brain tumors. The WHO classifies GBM as IDH WT Grade 4 astrocytoma. GBM originates from glial cells and is characterized by high genomic instability leading to a highly heterogenous presentation. Prognosis remains poor with an OS of 14-16 months following SoC and a 5 year survival rate of <10%.

Current SoC for GBM involves surgical resection followed by radiotherapy and chemotherapy with temozolomide (TMZ), an oral alkylating agent. Complete surgical

removal is difficult due to tumor infiltration and anatomical proximity to important brain regions. TMZ efficacy is limited by tumor resistance and systemic tox including myelosuppression.

While we could not find any large databases describing MTAP deletion prevalence in GBM, in one study, a sample of 423 cases showed 45.6% had MTAP deletion, while in another there were 33.5% MTAP deletion samples out of 167 GBMs.

In addition to Tango, preclinical work with navlimetostat (previously MRTX-1719) showed strong anti-tumor activity in MTAP-deleted glioma cells. This proved more difficult in vivo as the presence of non-malignant, MTAP-WT cells in the tumor microenvironment can metabolize extracellular MTA, reducing its local concentration and reducing the inhibition it applies to PRMT5.

Preclinical Data Supporting TNG456 Development

In a recent *Journal of Medicinal Chemistry* paper from May 2026 ([link](#)), Tango describes the development of TNG456. TNG456 showed no off-target activity against a panel of 40 methyltransferases other than PRMT5 and no other potential off-target binding against 78 known off-target assays. In cell viability assays with cell pairs that were either MTAP WT or MTAP-deleted, TNG456 showed an average of 55x selectivity for the MTAP-deleted cells.

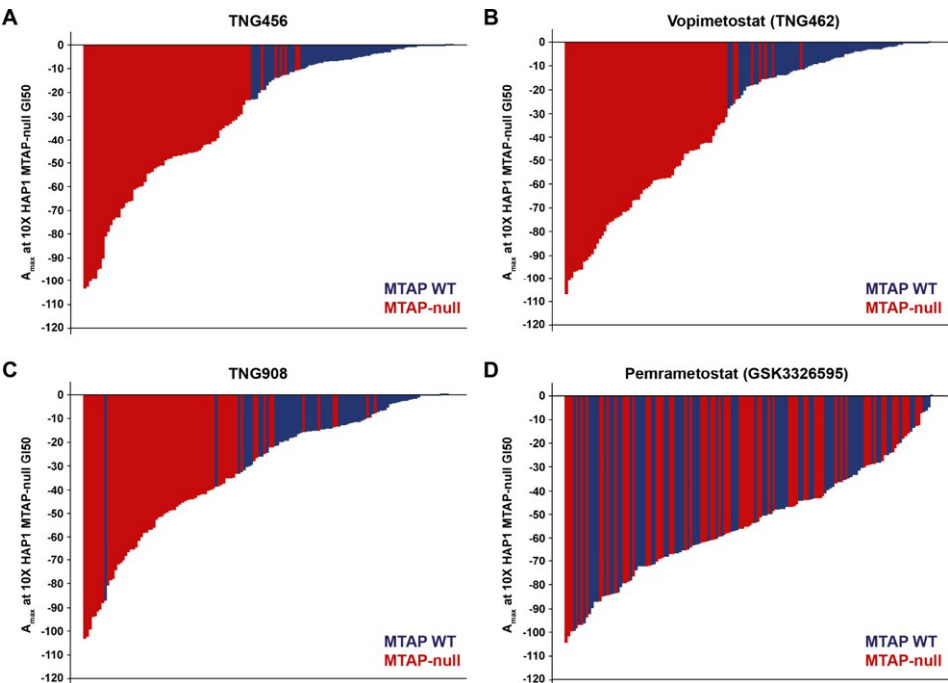
Figure 36: Cell viability with TNG456 in four MTAP-isogenic cell lines

Cell Line (Histology)	GI50 ± SD (µM)		Selectivity (fold)
	MTAP-null	MTAP WT	
LU99 (NSCLC)	0.040 ± 0.012	1.5 ± 0.16	38
LN18 (GBM)	0.054 ± 0.007	3.5 ± 1.2	65
HCT116 (CRC)	0.015 ± 0.005	0.98 ± 0.098	65
HAP1 (CML)	0.022 ± 0.005	1.1 ± 0.43	50
Average: 55x			

Source: Cottrell, Kevin M., et al. "The Discovery of TNG456: A Highly Potent, Selective, Brain-Penetrant MTA-Cooperative PRMT5 Inhibitor for the Treatment of MTAP-Deleted Cancers." *Journal of Medicinal Chemistry* (2026).

TNG456, vopimetostat and TNG908 were compared to a PRMT5i with non-MTA-cooperative MOA in a panel of 143 cell lines (50/50 MTAP WT/deleted) across multiple histologies. The data confirmed both vopimetostat and TNG456's high selectivity towards MTAP-deleted cells.

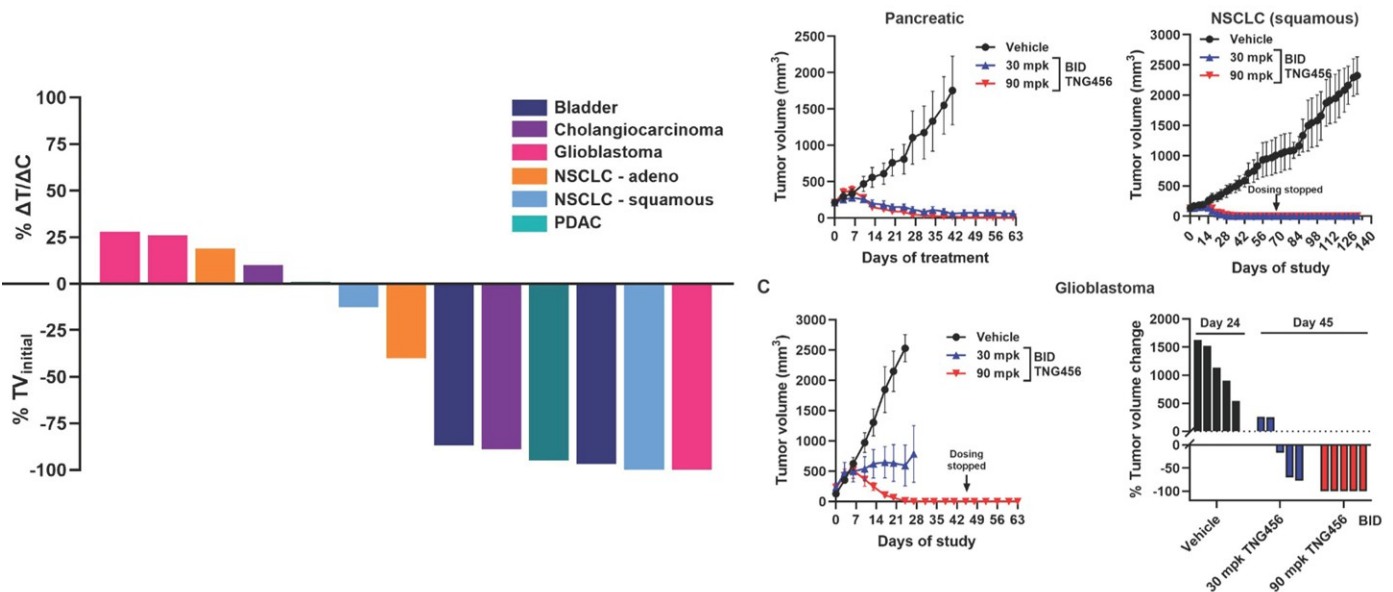
Figure 37: Comparison of cell killing in a 143-cancer cell line panel



Source: Cottrell, Kevin M., et al. "The Discovery of TNG456: A Highly Potent, Selective, Brain-Penetrant MTA-Cooperative PRMT5 Inhibitor for the Treatment of MTAP-Deleted Cancers." *Journal of Medicinal Chemistry* (2026).

TNG456 was also tested across 13 MTAP-deleted PDX models from tumors with a high frequency of MTAP-deletion. In a 90mpk BID dose mouse model, the best responses in terms of tumor growth inhibition were seen with the squamous NSCLC and GBM models.

Figure 38: TNG456 is efficacious in a panel of MTAP-null PDX models

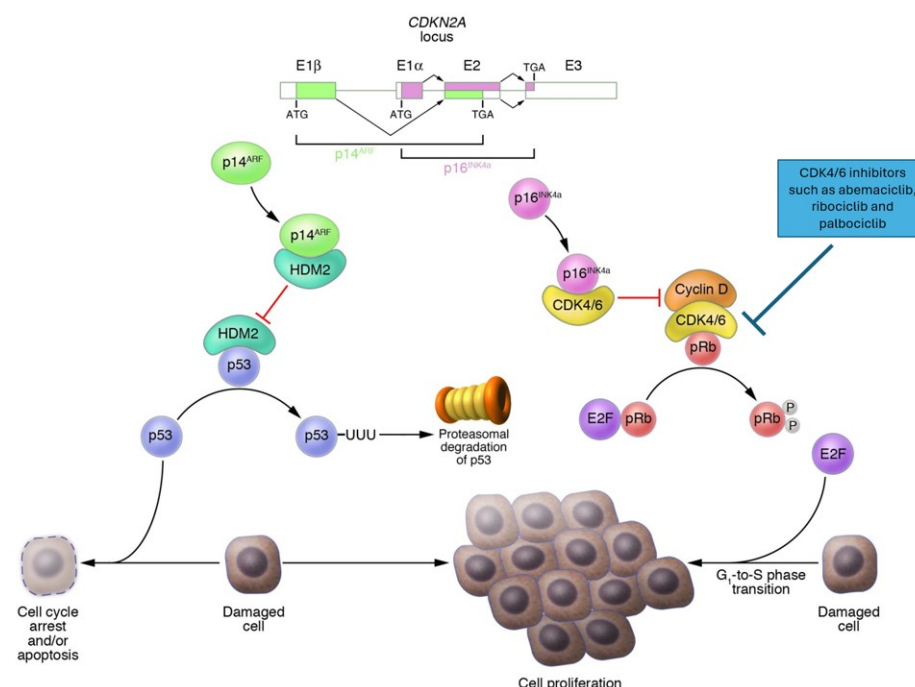


Source: Cottrell, Kevin M., et al. "The Discovery of TNG456: A Highly Potent, Selective, Brain-Penetrant MTA-Cooperative PRMT5 Inhibitor for the Treatment of MTAP-Deleted Cancers." *Journal of Medicinal Chemistry* (2026).

Overall, these assays support TNG456 monotherapy activity in MTAP-deleted tumors. As discussed earlier, since MTAP deletion is usually accompanied by CDKN2A deletion, these cancers may have increased CDK4/6 activity. The CDKN2A locus on chromosome 9p21 encodes two tumor suppressors, p16INK4a and p14ARF. Normally, p16INK4a sequesters CDK4 and CDK6, keeping pRb in an active, hypophosphorylated state. In the absence of p16INK4a, CDK4 and CDK6 bind to cyclin D and phosphorylate Rb which then releases E2F promoting the G1-to-S phase transition of the cell cycle.

This means that cells with the 9p21 deletion exhibit both MTAP deletion, rendering them sensitive to PRMT5 inhibition, and have a reliance on CDK4/6 making them sensitive to its inhibition. There are multiple approved CDK4/6 inhibitors, including palbociclib, ribociclib and abemaciclib. Since these are two discrete susceptibility pathways, similar to RAS, it makes sense that targeting both would be synergistic.

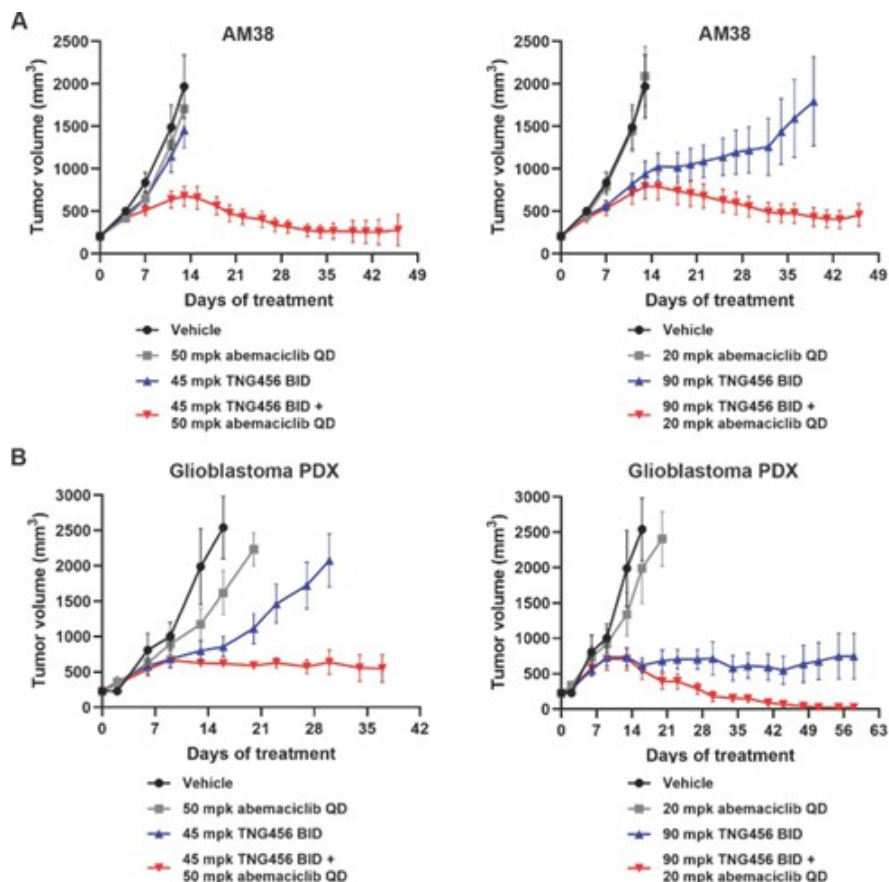
Figure 39: Genetic encoding and mechanism of action of tumor suppressors p16INK4a and p14ARF



Source: Chudnovsky, Yakov, Paul A. Khavari, and Amy E. Adams. "Melanoma genetics and the development of rational therapeutics." The Journal of Clinical Investigation 115.4 (2005): 813-824.

Based on this, Tango tested TNG456 in combination with abemaciclib, a brain-penetrant CDK4/6i, to see if it enhances anti-tumor activity. In the MTAP-deleted AM38 GBM CDX model, neither TNG456 or abemaciclib induced tumor regression, but the combo showed anti-tumor activity. Similarly, in a GBM PDX model, there was strong synergistic activity where the clinically relevant 90mpk BID TNG456 with a lower sub-therapeutic dose of abemaciclib led to a complete response. This was not repeated when using a sub-therapeutic dose of TNG456 with a clinical dose of abemaciclib. Importantly, abemaciclib is a CYP3A4 substrate, while TNG456 was found not to be, lowering the DDI risk.

Figure 40: Combination of TNG456 with abemaciclib drives a strong benefit in glioblastoma xenograft models.

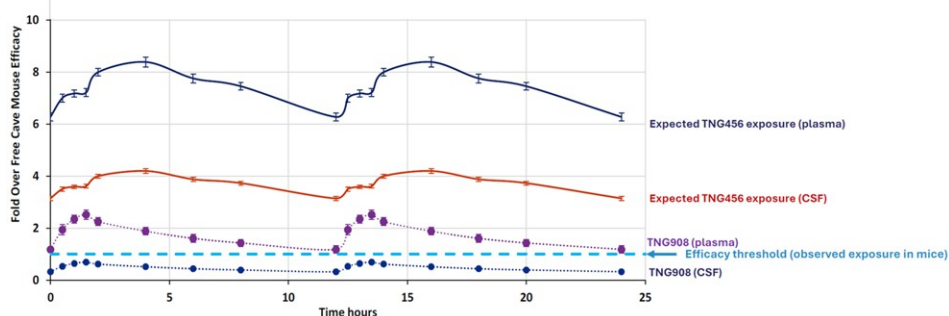


Source: Cottrell, Kevin M., et al. "The Discovery of TNG456: A Highly Potent, Selective, Brain-Penetrant MTA-Cooperative PRMT5 Inhibitor for the Treatment of MTAP-Deleted Cancers." *Journal of Medicinal Chemistry* (2026).

Tango evaluated TNG456 in preclinical animal models to assess PK and the ratio of unbound brain-to-plasma ratio ($K_{p,u}$) of the drug to assess CNS-penetration at acceptable levels. The predicted human half-life for TNG456 was ~6h suggesting a BID dosing regimen (vopimetostat is QD). These levels are projected to keep the concentration above the GI_{90} levels for MTAP-deleted cells while remaining below the GI_{50} for WT cells. Brain penetration was confirmed in both models, showing $K_{p,u}$ values of 0.59 (0.57-0.61) in cynomolgus monkeys and 0.64 (0.44-1.1) in beagle dogs.

Figure 41: Expected TNG456 exposure vs observed TNG908 exposure

TNG456 brain exposure modeled at 50% of plasma; expected exposure assumes dose proportional increase in exposure from 20 and 40 mg BID



Source: Corporate presentation November 2025

In summary, preclinical data shows that TNG456 has:

1. MTAP-null GI_{50} = 20 nM with 55-fold selectivity vs MTAP WT cells in the HAP1 cell line.
2. $K_{p,uu}$ ranging from 0.5 to 1.0 across preclinical species supporting its potential to achieve therapeutically relevant CNS exposure in patients.
3. A favorable CYP3A4 profile with no preclinical signals of CYP3A4 inhibition, suggesting a very low risk of clinical CYP3A4 mediated DDIs.
4. A preclinical DMPK profile predicting human exposure sufficient to maintain MTAP-null GI_{90} coverage in the brain at trough. It had an IC_{50} of 9.4 μ M representing nearly a 100-fold safety margin relative to the targeted MTAP-null GI_{90} of 100 nM.

Financial Outlook

Income Statement

We forecast FY26-28 GAAP Basic EPS of (\$1.30), (\$1.24), and \$(1.28), respectively.

Balance Sheet and Cash Flow

As of the date of this report, TNGX had ~\$947mn in cash and cash equivalents. We estimate operating expenses of ~\$208mn, ~\$229mn and ~\$252mn in 2026, 2027 and 2028, respectively, with current cash on hand providing sufficient runway until cash flow positive.

Valuation

Our Dec 2026 price target of \$46 is based on our DCF SOTP calculation. We assume stand-alone contributions from vopimetostat in 1L PDAC and 2L NSCLC as well as TNG456 contributions in GBM. Our \$46 price target consists of \$39/share for vopimetostat in 1L PDAC, ~\$1/share for vopimetostat in 2L NSCLC, \$0.4/share for TNG456 in GBM and the remainder for corporate expenses/pipeline and cash.

Figure 42: Valuation overview

SOTP Calculation		\$ / share	Key Model Assumptions	
Vopimetostat in 1L PDAC		\$ 38.8	Vopimetostat in 1L PDAC	
Vopimetostat in NSCLC		\$ 1.10	WACC	10%
TNG456 in GBM		\$ 0.4	Peak Penetration	60%
Pipeline & Corporate Expenses		\$ 0.0	Pricing in US	\$ 30,000
NOLs		\$ 0.3	GTN	15%
Net cash		\$ 4.9	Annual Price Increase (US)	4%
Total		\$ 45.6	Peak Sales (\$mn) - Risk-adjusted	\$ 3,485

Key Model Assumptions	
Vopimetostat in NSCLC	
WACC	11%
Peak Penetration	15%
Pricing in US	\$ 30,000
GTN	15%
Annual Price Increase (US)	4%
Peak Sales (\$mn) - Risk-adjusted	\$ 150

Key Model Assumptions	
TNG456 in GBM	
WACC	11%
Peak Penetration	30%
Pricing in US	\$ 30,000
GTN	15%
Annual Price Increase (US)	3%
Peak Sales (\$mn) - Risk-adjusted	\$ 96

Source: J.P. Morgan estimates.

Executive Team Overview

Malte Peters, MD, CEO – Dr. Peters has served as President and CEO of Tango Therapeutics since January 2026 and has served on the Tango board of directors since 2018. Most recently, Dr. Peters served as Chief Research and Development Officer of MorphoSys AG, a biopharmaceutical company, and prior to that served as its Chief Development Officer and a member of its management board since March 2017. Prior to MorphoSys, Dr. Peters served as Global Head of Clinical Development of the Biopharmaceuticals Business Unit at Sandoz International. Prior to this position, he served as Clinical Head and Site Head for Basel and East Hanover in Oncology Translational Medicine at Novartis. Dr. Peters held teaching appointments in Internal

Medicine and Biochemistry at the University of Mainz, Germany. Earlier in his career, he served as a Research Scientist at the Amgen Research Institute in Toronto, Canada, as Director of Cancer Research at Merck KGaA in Darmstadt, Germany, and as Medical Director at Micromet AG in Munich, Germany. Dr. Peters received his MD from the Freie Universität Berlin, Germany, and was trained at the Universities of Padova, Italy, and Bochum and Berlin, Germany. He received his habilitation in Internal Medicine from the University of Mainz, Germany.

Adam Crystal, MD, PhD, President of Research and Development – Dr. Crystal oversees clinical and development operations as President of Research and Development at Tango Therapeutics. Previously, Dr. Crystal served as CMO of C4 Therapeutics, where he oversaw the advancement of several targeted protein degraders into clinical trials in oncology patients. Prior to C4, he was at Novartis Institutes for Biomedical Research, most recently as Senior Director, where he led several early clinical development programs. Dr. Crystal also served as a medical oncologist at Massachusetts General Hospital, where he researched resistance mechanisms to targeted therapies. Dr. Crystal completed his Bachelor of Arts in neuroscience at the University of Pennsylvania and earned his MD and PhD in neuroscience from the University of Pennsylvania School of Medicine. He completed his internal medicine training at Massachusetts General Hospital and his fellowship in medical oncology at the Massachusetts General Hospital Cancer Center and the Dana-Farber Cancer Institute.

Matthew Gall, CFO – Mr. Gall joined Tango in April 2026. Most recently, Mr. Gall served as CFO of Kalaris Therapeutics. Prior to that, he served as CFO of iTeos Therapeutics. He also previously held positions of increasing responsibility across the finance and business development functions at Sarepta Therapeutics, Celgene Corporation, and Gilead Sciences. Matt holds a Bachelor of Science in Business Administration from Bowling Green State University and a Master of Business Administration from The University of Chicago.

Jannik N. Andersen, PhD, Chief Scientific Officer – Brings extensive expertise in basic and applied cancer research to his role overseeing the integration of functional genomics, drug discovery biology, and translational and computational biology. Previously, Dr. Andersen worked across the translational continuum in larger pharmaceutical companies including Novo Nordisk and Merck, in the biotech industry at Xtuit and Xios Pharmaceuticals, and in major university hospitals, where he held director-level positions at the Dana-Farber Cancer Institute, MD Anderson Cancer Center, University of Texas. Dr. Andersen brings a multidisciplinary background, first graduating as an Erasmus Scholar from Institute Polytechnique de Grenoble before earning a Master of Science in chemical engineering from the Technical University of Denmark and a doctorate in human biology from the Faculty of Medicine, University of Copenhagen. He completed his PhD thesis at Novo Nordisk and conducted his post-doctoral studies at Cold Spring Harbor Laboratory, New York, focusing on molecularly targeted therapies.

Financial Statements

Figure 43: Income statement

Income Statement (\$ in mn)	2021A	2022A	2023A	2024A	2025A	1Q26A	2Q26E	3Q26E	4Q26E	2026E	2027E	2028E
Product Revenue	-	-	-	-	-	-	-	-	-	-	-	-
Vopinetotal	-	-	-	-	-	-	-	-	-	-	-	-
THQ456	-	-	-	-	-	-	-	-	-	-	-	-
Collaboration and License Revenue	37.04	34.98	36.5	42.1	62.4	-	-	-	-	-	-	-
Total Revenues	37.04	24.86	36.5	42.1	62.4	-	-	-	-	-	-	-
COGS	-	-	-	-	-	-	-	-	-	-	-	-
R&D	77.64	105.91	115.2	143.9	132.2	33.5	34.9	36.3	37.7	142.4	156.6	172.3
% q/q or % y/y	-	26%	9%	25%	-8%	4%	4%	4%	4%	8%	10%	10%
% q/q or % y/y	-	26%	3%	13%	-22%	4%	13%	6%	6%	5%	-3%	-16%
SG&A	17.60	30.83	35.5	43.7	41.5	15.2	16.0	16.8	17.6	65.7	72.2	79.5
% q/q or % y/y	-	71%	16%	23%	-5%	56%	5%	5%	5%	58%	10%	10%
% q/q or % y/y	-	57%	17%	16%	-1%	13%	5%	5%	5%	16%	16%	17%
Other	-	-	-	-	-	-	-	-	-	-	-	-
Total Operating Expenses	95.23	135.93	150.70	187.66	173.67	48.8	50.9	53.1	55.4	208.1	228.9	251.8
Operating Income	(58.19)	(111.07)	(114.17)	(145.60)	(111.29)	(48.8)	(50.9)	(53.1)	(55.4)	(208.1)	(228.9)	(251.8)
Interest income	0.50	1.46	6.62	7.89	5.6	2.2	2.4	2.6	2.6	9.7	9.7	9.7
Other income, net	(0.25)	1.49	5.94	7.61	4.1	1.1	-	-	-	1.1	1.1	1.1
Change in fair value of redeemable preferred stock liability	-	-	-	-	-	-	-	-	-	-	-	-
Pretax Income	(57.94)	(108.12)	(101.61)	(130.09)	(101.59)	(46.5)	(48.5)	(50.5)	(52.8)	(197.3)	(216.1)	(241.0)
Provision for income taxes	(0.29)	(0.05)	(0.13)	(0.21)	(0.0)	(0.0)	-	-	-	(0.0)	-	-
Net Income	(58.24)	(108.18)	(101.74)	(130.30)	(101.59)	(46.5)	(48.5)	(50.5)	(52.8)	(197.3)	(216.1)	(241.0)
Unrealized (loss) gain on marketable securities	(0.78)	(2.94)	3.9	0.2	(0.1)	(0.3)	-	-	-	(0.3)	-	-
Comprehensive loss	(59.02)	(111.12)	(97.9)	(130.2)	(101.7)	(46.8)	(48.5)	(50.5)	(52.8)	(197.6)	(216.1)	(241.0)
Basic EPS	(0.54)	(1.22)	(1.60)	(1.74)	(1.22)	(0.32)	(0.31)	(0.30)	(0.31)	(1.87)	(2.04)	(2.28)
Diluted EPS	(0.55)	(0.91)	(1.03)	(1.19)	(0.88)	(0.32)	(0.31)	(0.30)	(0.31)	(1.30)	(1.24)	(1.20)
Weighted Average Basic Shares Outstanding	62.1	87.6	94.6	109.2	116.2	143.6	155.0	166.6	168.2	152.5	175.6	188.6

Source: J.P. Morgan estimates, Company data.

Figure 44: Balance sheet

Balance Sheet (\$mn)	2020A	2021A	2022A	2023A	2024A	2025A	2026E	2027E	2028E
Current Assets:									
Cash and cash equivalents	28.4	142.7	60.0	66.4	69.5	112.3	539.4	374.6	187.0
Marketable securities	161.9	342.5	306.2	270.5	188.4	230.9	230.9	230.9	230.9
Accounts receivable	2.0	2.0	2.0	-	-	-	-	-	-
Restricted cash	-	0.6	0.6	0.9	-	0.4	0.4	0.4	0.4
Prepaid expenses and other current assets	1.3	4.5	6.6	8.8	8.4	10.2	10.2	10.2	10.2
Total Current Assets	193.6	492.3	375.3	346.5	266.3	353.8	780.9	616.1	428.4
Property and equipment, net	3.8	4.8	10.9	9.9	8.1	6.9	5.6	4.8	4.3
Operating lease right-of-use assets	7.5	1.3	48.9	43.5	39.5	35.6	31.7	27.7	23.7
Restricted cash, net of current portion	2.3	1.7	3.4	2.6	2.6	2.1	2.1	2.1	2.1
Other assets	0.0	0.0	0.0	0.0	0.0	0.3	0.3	0.3	0.3
TOTAL ASSETS	207.3	500.2	436.5	402.6	316.5	398.7	820.6	651.1	458.9
Current Liabilities:									
Accounts Payable	1.8	3.2	4.5	2.8	1.6	1.2	1.4	1.6	1.7
% of OpEx	-	3.39%	3.28%	1.85%	0.85%	0.68%	0.68%	0.68%	0.68%
Accrued expenses	6.1	9.9	17.5	15.4	16.5	17.8	21.3	23.4	25.7
% of OpEx	-	10.38%	12.87%	10.22%	8.79%	10.23%	10.23%	10.23%	10.23%
Operating lease liability	1.0	1.5	1.8	2.1	2.5	2.7	2.7	2.7	2.7
Deferred revenue, current	32.0	28.0	31.8	25.7	17.6	-	-	-	-
Income tax payable	-	0.1	0.0	-	-	-	-	-	-
Total Current Liabilities	40.9	40.690	55.5	45.9	38.2	21.7	25.4	27.7	30.2
Operating lease liabilities, net of current portion	6.9	-	39.4	36.8	34.0	30.8	30.8	30.8	30.8
Deferred revenue, net of current portion	120.8	114.7	92.1	66.7	44.8	-	-	-	-
Other long-term liabilities	0.0	-	-	-	-	-	-	-	-
TOTAL LIABILITIES	168.7	155.4	187.0	149.5	117	53	56.3	58.5	61.0
Redeemable convertible preferred stock	-	-	-	-	-	-	-	-	-
Common Stock	0.0	0.1	0.1	0.1	0.1	0.1	0.1	0.1	0.1
Additional paid in capital	141.6	506.8	522.6	624.1	700.6	949.0	1,595.3	1,641.6	1,687.8
Accumulated Other comprehensive income	0.0	(0.8)	(3.7)	0.2	0.3	0.2	0.2	0.2	0.2
Accumulated deficit	(103.1)	(161.3)	(269.5)	(371.3)	(501.6)	(603.2)	(831.3)	(1,049.4)	(1,290.3)
TOTAL STOCKHOLDERS' EQUITY	38.6	344.7	249.5	253.1	199.5	346.2	764.3	592.5	397.9
TOTAL LIABILITIES AND SH EQUITY	207.3	500.2	436.5	402.6	316.5	398.7	820.6	651.1	458.9

Source: J.P. Morgan estimates, Company data.

Figure 45: Cash flow statement

Cash Flows (\$mn)	2020A	2021A	2022A	2023A	2024A	2025A	2026E	2027E	2028E
Net Income	(52.0)	(58.2)	(108.2)	(101.7)	(130.3)	(101.6)	(197.3)	(218.1)	(241.0)
D&A	0.7	0.9	1.6	2.4	2.5	2.3	2.3	1.9	1.6
% of PP&E	18.78%	18.56%	14.77%	24.37%	30.84%	33.21%	33.21%	33.21%	33.21%
Noncash operating lease expense	0.9	1.1	2.4	3.6	3.7	4.0	4.0	4.0	4.0
SBC	1.8	7.8	14.2	19.1	28.9	26.4	46.3	46.3	46.3
Accretion on marketable securities	-	-	-	(3.4)	(4.6)	(2.7)	-	-	-
Other, net	(0.0)	0.3	(0.7)	0.0	0.0	(0.1)	-	-	-
Changes in Operating Assets and Liabilities :									
Accounts receivable	(2.0)	-	-	2.0	-	-	-	-	-
Prepaid expenses and other current assets	(0.1)	(3.2)	(2.1)	(2.2)	0.4	(1.8)	-	-	-
Other long-term assets	(0.0)	(0.1)	(10.0)	(0.0)	0.0	(0.3)	-	-	-
Accounts payable	1.2	1.2	1.1	(1.7)	(1.2)	(0.8)	0.2	0.1	0.2
Accrued expenses and other liabilities	1.2	3.8	7.6	(2.0)	1.1	1.1	3.5	2.1	2.3
Operating lease liabilities	(0.7)	(1.1)	1.7	(2.4)	(2.1)	(3.0)	-	-	-
Deferred revenue	119.1	(12.0)	(16.9)	(31.5)	(30.0)	(62.4)	-	-	-
Net Cash Used in Operating Activities	70.1	(59.5)	(109.1)	(118.0)	(131.5)	(138.9)	(141.0)	(163.7)	(186.6)
Purchase of PP&E	(1.1)	(1.8)	(7.7)	(1.5)	(0.8)	(1.0)	(1.0)	(1.0)	(1.0)
Sales of marketable securities	63.2	190.8	242.6	353.5	327.8	224.6	-	-	-
Purchases of marketable securities	(207.6)	(372.4)	(208.5)	(310.6)	(240.9)	(264.5)	-	-	-
Net cash provided by investing activities	(145.5)	(183.4)	26.4	41.4	86.1	(40.9)	(1.0)	(1.0)	(1.0)
Proceeds from issuance of common stock and pre-fund	-	-	-	80.0	41.7	212.4	600.0	-	-
Proceeds from issuance of common stock upon exercise	80.9	357.3	1.6	1.9	5.9	10.1	-	-	-
Return of capital from a stockholder - related party	-	-	-	0.7	-	-	-	-	-
Payment of transaction costs	-	-	-	(0.2)	-	(0.0)	(33.0)	-	-
Net Cash provided by financing activities	80.9	357.3	1.6	82.4	47.7	222.5	567.0	-	-
Net decrease in cash and cash equivalents	5.5	114.4	(81.1)	5.8	2.3	42.7	425.0	(164.7)	(187.7)
Cash and equivalents at beginning of year	25.2	30.7	145.0	64.0	69.8	72.1	114.8	539.8	375.1
Cash and equivalents at end of year	30.7	145.0	64.0	69.8	72.1	114.8	539.8	375.1	187.4

Source: J.P. Morgan estimates, Company data.

Investment Thesis, Valuation and Risks

Tango Therapeutics (*Overweight; Price Target: \$46.00*)

Investment Thesis

We have an Overweight rating on TNGX shares. We see TNGX set up well following the strong combinatorial data for its lead PRMT5 inhibitor vopimetostat with RAS inhibitors in 2L+ PDAC which supports its planned Ph 3 clinical trial plans in 1L PDAC. While data is early, the unprecedented efficacy in such a difficult indication supports its penetration into the large portion of PDAC patients with MTAP-deletion. Beyond vopimetostat, Tango's CNS-penetrant PRMT5 inhibitor, TNG456, is currently in Ph 1/2 for the very difficult to treat glioblastoma and provides a meaningful virtual "call option" that we think is not reflected in shares today.

Valuation

Our Dec 2026 price target of \$46 is based on our DCF SOTP calculation. We assume stand-alone contributions from vopimetostat in 1L PDAC and 2L NSCLC as well as TNG456 contributions in GBM. Our \$46 price target consists of \$39/share for vopimetostat in 1L PDAC, ~\$1/share for vopimetostat in 2L NSCLC, \$0.4/share for TNG456 in GBM and the remainder for corporate expenses/pipeline and cash.

Risks to Rating and Price Target

Key risks to our rating and price target include 1) the inability to replicate vopimetostat's combo efficacy with daraxonrasib in a larger Ph 3 trial, 2) inability to differentiate vs competitor PRMT5is from larger established pharma, 3) clinical and regulatory risks, and 4) cash needs to bring multiple assets forward.

Tango Therapeutics: Summary of Financials

Income Statement - Annual						Income Statement - Quarterly					
	FY24A	FY25A	FY26E	FY27E	FY28E		1Q26A	2Q26E	3Q26E	4Q26E	
Revenue	42	62	0	0	-	Revenue	0A	0	0	0	0
COGS	0	0	0	0	-	COGS	0A	0	0	0	0
Gross profit	-	-	-	-	-	Gross profit	-	-	-	-	-
SG&A	(44)	(42)	(66)	(72)	-	SG&A	(15)A	(16)	(17)	(18)	-
Adj. EBITDA	(143)	(109)	(206)	(229)	-	Adj. EBITDA	(49)A	(51)	(53)	(55)	-
D&A	(2)	(2)	(2)	0	-	D&A	-	-	-	-	-
Adj. EBIT	(146)	(111)	(208)	(229)	-	Adj. EBIT	(49)A	(51)	(53)	(55)	-
Net Interest	8	6	10	10	-	Net Interest	2A	2	3	3	-
Adj. PBT	(130)	(102)	(197)	(218)	-	Adj. PBT	(46)A	(48)	(50)	(53)	-
Tax	(0)	(0)	(0)	0	-	Tax	(0)A	0	0	0	-
Minority Interest	-	-	-	-	-	Minority Interest	-	-	-	-	-
Adj. Net Income	(130)	(102)	(198)	(218)	-	Adj. Net Income	(46)A	(48)	(50)	(53)	-
Reported EPS	(1.19)	(0.88)	(1.30)	(1.24)	-	Reported EPS	(0.32)A	(0.31)	(0.30)	(0.31)	-
Adj. EPS	(1.19)	(0.88)	(1.30)	(1.24)	-	Adj. EPS	(0.32)A	(0.31)	(0.30)	(0.31)	-
DPS	-	-	-	-	-	DPS	-	-	-	-	-
Payout ratio	-	-	-	-	-	Payout ratio	-	-	-	-	-
Shares outstanding	109	116	152	176	-	Shares outstanding	144A	155	167	168	-
Balance Sheet & Cash Flow Statement						Ratio Analysis					
	FY24A	FY25A	FY26E	FY27E	FY28E		FY24A	FY25A	FY26E	FY27E	FY28E
Cash and cash equivalents	70	112	539	375	-	Gross margin	-	-	-	-	-
Accounts receivable	0	0	0	0	-	EBITDA margin	(340.1%)	(174.7%)	-	-	-
Inventories	-	-	-	-	-	EBIT margin	(346.1%)	(178.4%)	-	-	-
Other current assets	8	11	11	11	-	Net profit margin	(309.4%)	(163.0%)	-	-	-
Current assets	266	354	781	616	-	ROE	(57.5%)	(37.3%)	(35.6%)	(32.1%)	-
PP&E	8	7	6	5	-	ROA	(36.2%)	(28.4%)	(32.4%)	(29.6%)	-
LT investments	-	-	-	-	-	ROCE	(64.4%)	(40.8%)	(37.5%)	(33.7%)	-
Other non current assets	3	2	2	2	-	SG&A/Sales	104.0%	66.5%	-	-	-
Total assets	316	399	821	651	-	Net debt/equity	-	-	-	-	-
Short term borrowings	-	-	-	-	-	P/E (x)	NM	NM	NM	NM	-
Payables	2	1	1	2	-	P/BV (x)	16.2	9.9	5.9	8.8	-
Other short term liabilities	37	20	24	26	-	EV/EBITDA (x)	NM	NM	NM	NM	-
Current liabilities	38	22	25	28	-	Dividend Yield	-	-	-	-	-
Long-term debt	-	-	-	-	-	Sales/Assets (x)	0.1	0.2	0.0	0.0	-
Other long term liabilities	79	31	31	31	-	Interest cover (x)	18.1	19.4	21.2	23.6	-
Total liabilities	117	53	56	59	-	Operating leverage	181.4%	(48.8%)	(87.0%)	-	-
Shareholders' equity	200	346	764	593	-	Revenue y/y Growth	15.2%	48.3%	(100.0%)	-	-
Minority interests	-	-	-	-	-	EBITDA y/y Growth	28.0%	(23.8%)	88.8%	11.2%	-
Total liabilities & equity	316	399	821	651	-	Tax rate	(0.2%)	(0.0%)	(0.0%)	0.0%	-
BVPS	1.83	2.98	5.01	3.38	-	Adj. Net Income y/y Growth	33.0%	(21.9%)	94.3%	10.4%	-
y/y Growth	(31.7%)	63.1%	68.2%	(32.7%)	-	EPS y/y Growth	15.2%	(26.5%)	48.0%	(4.1%)	-
Net debt/(cash)	-	-	-	-	-	DPS y/y Growth	-	-	-	-	-
Cash flow from operating activities	(132)	(139)	(141)	(164)	-						
o/w Depreciation & amortization	2	2	2	2	-						
o/w Changes in working capital	(32)	(67)	4	2	-						
Cash flow from investing activities	86	(41)	(1)	(1)	-						
o/w Capital expenditure	(1)	(1)	(1)	(1)	-						
as % of sales	1.8%	1.7%	-	-	-						
Cash flow from financing activities	48	223	567	0	-						
o/w Dividends paid	-	-	-	-	-						
o/w Net debt issued/(repaid)	-	-	-	-	-						
Net change in cash	2	43	425	(165)	-						
Adj. Free cash flow to firm	(140)	(146)	(152)	(174)	-						
y/y Growth	11.1%	3.9%	4.3%	15.0%	-						

Source: Company reports and J.P. Morgan estimates.

Note: \$ in millions (except per-share data).Fiscal year ends Dec. o/w - out of which

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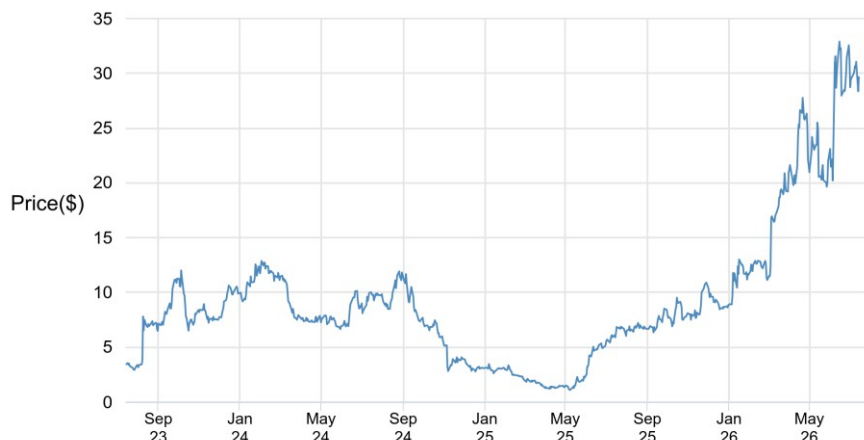
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Tango Therapeutics (TNGX, TNGX US) Price Chart



Source: Bloomberg Finance L.P. and J.P. Morgan; price data adjusted for stock splits and dividends.

The chart(s) show J.P. Morgan's continuing coverage of the stocks; the current analysts may or may not have covered it over the entire period. J.P. Morgan ratings or designations: OW = Overweight, N= Neutral, UW = Underweight, NR = Not Rated

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